GSSSB · STAFF NURSE EXAM

Child Health Nursing MCQ Bank

640 MCQs · GSSSB pattern · Answer key with explanation



19 Sections | 640 MCQs | Direct + Priority + Scenario | Match & Assertion-Reason | Calculations | Explanations

Hinglish explanation + English exam terms



Sister Asha
Aapki Nursing teacher

MCQ Bank – Sections

1. **Introduction & Child Health Programmes**
   30 MCQs · Age groups, indicators, child rights & laws, RBSK, RKSK, HBNC, HBYC, ICDS, Mission Indradhanush, important days
2. **Growth & Development**
   30 MCQs · Principles, anthropometry & formulas, fontanelles, dentition, milestones, Erikson/ Freud/ Piaget, play
3. **Nutrition, IYCF & Malnutrition**
   30 MCQs · Breastfeeding, complementary feeding, SAM, marasmus vs kwashiorkor, WHO 10 steps, vitamin & mineral deficiencies, anaemia
4. **Immunization**
   30 MCQs · Vaccine types, UIP schedule, dose/ route/ site, cold chain, VVM, open vial policy, AEFI, contraindications
5. **Hospitalised Child, Procedures & Drug Calculations**
   30 MCQs · Separation anxiety, pain scales, vital signs, medication administration, dose rules, IV fluids, restraints, specimen collection
6. **High-Risk Newborn & Neonatal Disorders**
   30 MCQs · Preterm, LBW, thermoregulation, KMC, feeding, jaundice & phototherapy, sepsis, asphyxia, RDS, hypoglycaemia, NEC, ROP, SNCU
7. **IMNCI, Diarrhoea & Dehydration**
   30 MCQs · IMNCI process, danger signs, fast breathing, colour classification, diarrhoea types, dehydration, Plan A/B/C, ORS, zinc, worms
8. **Respiratory Disorders**
   30 MCQs · Airway anatomy, tonsillitis, otitis media, croup, epiglottitis, bronchiolitis, pneumonia, asthma, foreign body, cystic fibrosis, TB
9. **Cardiovascular Disorders**
   30 MCQs · CHD types, VSD, ASD, PDA, coarctation, TOF, TGA, tet spell, CHF & digoxin, rheumatic fever, Kawasaki
10. **Gastrointestinal Disorders**
    30 MCQs · Cleft lip/ palate, TEF, pyloric stenosis, intussusception, Hirschsprung, anorectal malformation, abdominal wall defects, coeliac, biliary atresia
11. **Genitourinary & Renal Disorders**
    30 MCQs · Nephrotic syndrome, AGN, UTI, Wilms tumour, hypospadias, undescended testis, phimosis, enuresis, HUS
12. **Neurological Disorders**
    30 MCQs · Hydrocephalus, VP shunt, spina bifida, meningitis, febrile seizures, epilepsy, status epilepticus, cerebral palsy, Reye's, DMD
13. **Haematology & Oncology**
    30 MCQs · Anaemia, thalassaemia, sickle cell, G6PD, haemophilia, ITP, leukaemia, retinoblastoma, neuroblastoma, chemotherapy nursing

14 **Endocrine, Metabolic & Genetic Disorders**
30 MCQs · Congenital hypothyroidism, T1DM, DKA, hypoglycaemia, CAH, PKU, galactosaemia, Down syndrome, Turner, Klinefelter, inheritance

15 **Communicable Diseases**
30 MCQs · Measles, chickenpox, rubella, mumps, roseola, diphtheria, pertussis, tetanus, polio, typhoid, dengue, malaria, rabies, HIV, scabies

16 **Musculoskeletal Disorders & Burns**
30 MCQs · CTEV, DDH, fractures, traction, osteomyelitis, scoliosis, JIA, burns – TBSA, Parkland, nursing

17 **Behavioural Disorders, Child Abuse & Poisoning**
30 MCQs · Behaviour problems, ADHD, autism, intellectual disability, child abuse, POCSO, accidents, SIDS, poisoning antidotes, adolescent health

18 **Paediatric Emergencies & Life Support**
30 MCQs · ETAT, paediatric CPR, choking, shock, emergency drug doses, oxygen devices, raised ICP, anaphylaxis

19 **GSSSB Pattern Mock Test – 100 MCQs (Full Child Health Syllabus)**
100 MCQs · 100 questions, 100 marks, 0.25 negative marking – complete Child Health (Paediatric) Nursing

Level 1 – Direct: facts, values, definitions

Level 2 – Priority: "first / best / immediate" action

Level 3 – Advanced: scenario, NOT/EXCEPT, match, A-R, calculation



MCQ kaise solve karein?

Pehle khud answer socho, phir options padho. Har section ke baad answer key aur explanation hai, galti wale questions ka number apni notebook me likho. Exam me galat answer par **0.25 marks** kat sakte hain, isliye bina soche guess mat karo.

1 Introduction & Child Health Programmes

Level 1 – Direct

1. Neonatal period kitne din ka hota hai?
   (a) 1 year (b) 7 days (c) 28 days (d) 42 days
2. Toddler age group –
   (a) 0–1 yr (b) 1–3 yrs (c) 3–6 yrs (d) 6–12 yrs
3. WHO ke anusar adolescent age –
   (a) 10–19 yrs (b) 12–18 yrs (c) 13–19 yrs (d) 15–24 yrs
4. Community health status ka sabse sensitive indicator –
   (a) IMR (b) Crude death rate (c) Birth rate (d) MMR
5. Neonatal death ka sabse common cause (India) –
   (a) Prematurity / LBW (b) Tetanus (c) Measles (d) Diarrhoea
6. Child helpline number –
   (a) 104 (b) 108 (c) 181 (d) 1098
7. ICDS kab launch hua?
   (a) 1 April 2005 (b) 2 October 1975 (c) 15 August 1980 (d) 26 January 1985
8. RBSK kis cheez par focus karta hai?
   (a) Only immunization (b) Family planning (c) Adolescent health (d) 4 D's – defects, deficiencies, diseases, developmental delays
9. RKSK ka target age group –
   (a) 0–5 yrs (b) 6–9 yrs (c) 10–19 yrs (d) 20–24 yrs
10. Mission Indradhanush launch date –
    (a) 1 January 2016 (b) 2 October 2014 (c) 14 November 2015 (d) 25 December 2014
11. India ko WHO ne polio-free kab certify kiya?
    (a) 2 October 2012 (b) 7 April 2015 (c) 13 January 2011 (d) 27 March 2014
12. Children's Day (India) –
    (a) 5 September (b) 14 November (c) 20 November (d) 24 January
13. UN Convention on Rights of the Child (UNCRC) adopt hua –
    (a) 1979 (b) 1989 (c) 1992 (d) 2000
14. POCSO Act kis saal bana?
    (a) 2005 (b) 2012 (c) 2015 (d) 2019
15. RTE Act ke antargat free education ki age –
    (a) 0–14 yrs (b) 6–14 yrs (c) 6–18 yrs (d) 3–6 yrs
16. HBYC me ASHA ke home visits kab hote hain?
    (a) Every month till 5 yrs (b) Only at 9 months (c) Day 3, 7, 14 (d) 3, 6, 9, 12, 15 months
17. ORS Day –
    (a) 1 August (b) 14 November (c) 16 March (d) 29 July
18. National Deworming Day –
    (a) 1 June (b) 7 April (c) 10 February & 10 August (d) 16 May

Level 2 – Priority & Application

19. Institutional delivery ke baad HBNC me ASHA kitne home visits karti hai?
(a) 3 (b) 6 (c) 7 (d) 10

20. Paediatric nurse hospital me child ke adhikar (rights) ke liye bolti hai. Ye kaunsa role hai?
(a) Advocate (b) Case finder (c) Manager (d) Researcher

21. ICDS ki 6 services me kaun NAHI hai?
(a) Free surgery (b) Supplementary nutrition (c) Pre-school education (d) Immunization

22. Nurse ko 10 saal ke bachche ke saath sexual abuse ka shak hota hai. POCSO ke under nurse kya karegi?
(a) Parents se pooch kar chhod degi (b) Police/ authorities ko report karna mandatory hai (c) Sirf doctor ko batayegi (d) Kuch nahi

23. Family-centred care ka core principle –
(a) Sirf doctor decide kare (b) Family ko care ka partner manna (c) Visiting band (d) Child ko family se alag rakhna

24. Atraumatic care ka example –
(a) Venipuncture se pehle EMLA cream lagana (b) Child ko bina bataye injection (c) Parents ko bahar bhejna (d) Restraint use hamesha

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

25. Match: P. ICDS Q. RBSK R. RKSK S. HBNC — with 1. Adolescent health 2. Anganwadi 3. ASHA newborn visits 4. 4 D's screening
(a) P-2, Q-1, R-4, S-3 (b) P-2, Q-4, R-1, S-3 (c) P-4, Q-2, R-1, S-3 (d) P-3, Q-4, R-1, S-2

26. Ek area me 1 saal me 5000 live births aur 140 infant deaths huye. IMR –
(a) 14 (b) 28 (c) 56 (d) 140

27. Assertion (A): IMR community health ka sensitive indicator hai. Reason (R): Infant deaths environment, nutrition aur health services se turant prabhavit hote hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

28. Kaun sa jodi galat hai?
(a) World Prematurity Day – 17 Nov (b) National Girl Child Day – 24 Jan (c) ORS Day – 14 Nov (d) Universal Children's Day – 20 Nov

29. SDG 3.2 ke anusar 2030 tak U5MR target –
(a) ≤ 12 (b) ≤ 25 (c) ≤ 40 (d) ≤ 70

30. Perinatal period –
(a) 28 wks gestation se birth ke 7 din baad tak (b) Birth se 1 yr (c) Conception se birth (d) Birth se 28 din

Answer Key & Explanation

1-c 0–28 days.
2-b 1–3 yrs.
3-a WHO adolescent 10–19 yrs.
4-a IMR – most sensitive indicator.
5-a Prematurity/ LBW.
6-d CHILDLINE 1098.
7-b 2 Oct 1975.
8-d RBSK – 4 D's.
9-c RKSK – 10–19 yrs.
10-d 25 Dec 2014.
11-d 27 March 2014 (last case 13 Jan 2011).
12-b 14 Nov – Nehru jayanti.

13-b 20 Nov 1989; India ratified 1992.
14-b POCSO 2012.
15-b 6–14 yrs.
16-d HBYC – 3, 6, 9, 12, 15 months.
17-d 29 July.
18-c 10 Feb & 10 Aug.
19-b Day 3, 7, 14, 21, 28, 42 = 6 visits.
20-a Advocate.
21-a Surgery ICDS service nahi.
22-b POCSO – mandatory reporting.
23-b Family partner in care.
24-a EMLA – pain kam, atraumatic.
25-b Sahi.
26-b 140 ÷ 5000 × 1000 = 28.
27-a Dono sahi, R reason hai.
28-c ORS Day 29 July; 14 Nov Children's Day.
29-b U5MR ≤ 25, NMR ≤ 12.
30-a Perinatal – 28 wks to 7 days.

2 Growth & Development

Level 1 – Direct

1. Birth weight kab double hota hai?
   (a) 1 year (b) 2 years (c) 3 months (d) 5 months
2. Birth weight kab triple hota hai?
   (a) 1 year (b) 2 years (c) 3 years (d) 6 months
3. 1 saal par length approx –
   (a) 60 cm (b) 75 cm (c) 90 cm (d) 100 cm
4. Head aur chest circumference barabar kab hote hain?
   (a) 1 year (b) 6 months (c) 3 years (d) Birth
5. Anterior fontanelle kab band hota hai?
   (a) 3 years (b) 6–8 weeks (c) 6 months (d) 12–18 months
6. Pehla daant aam taur par kaunsa aata hai?
   (a) Upper lateral incisor (b) Lower central incisor (c) First molar (d) Upper canine
7. Sabhi 20 milk teeth kab tak aa jate hain?
   (a) 1 year (b) 2½ years (c) 5 years (d) 18 months
8. Social smile kab aati hai?
   (a) 6 months (b) 4 months (c) Birth (d) 6–8 weeks
9. Neck holding (head control) –
   (a) 1 month (b) 3 months (c) 6 months (d) 9 months
10. Bina support baithna –
    (a) 4 months (b) 6 months (c) 8 months (d) 12 months
11. Pincer grasp –
    (a) 4 months (b) 6 months (c) 9–10 months (d) 15 months
12. Bachcha akela chalna kab shuru karta hai?
    (a) 2 years (b) 9 months (c) 12–13 months (d) 18 months
13. Circle copy karna –
    (a) 2 years (b) 3 years (c) 4 years (d) 5 years
14. Tricycle chalana –
    (a) 2 years (b) 3 years (c) 4 years (d) 5 years
15. Erikson ke anusar infancy ka stage –
    (a) Autonomy vs Shame (b) Trust vs Mistrust (c) Industry vs Inferiority (d) Initiative vs Guilt
16. Piaget ke anusar object permanence kis stage me?
    (a) Concrete operational (b) Formal operational (c) Sensorimotor (d) Preoperational
17. Toddler kaunsa play karta hai?
    (a) Parallel (b) Competitive team (c) Solitary (d) Cooperative
18. Freud ke anusar toddler stage –
    (a) Oral (b) Latency (c) Phallic (d) Anal

Level 2 – Priority & Application

19. 9 mahine ka bachcha sahara lekar khada hota hai, "mama" bolta hai aur bye-bye karta hai. Nurse ka assessment –
(a) Delayed development (b) Normal development (c) Autism (d) Advanced

20. Mother ko 6 mahine ke bachche ke liye safe toy suggest karna hai –
(a) Coins (b) Small beads (c) Balloon (d) Large bright rattle / soft toy

21. Preschool child ke liye best toy –
(a) Doctor set / dolls (dramatic play) (b) Mobile hanging (c) Chess (d) Rattle

22. 18 mahine ka bachcha abhi tak nahi chalta. Nurse –
(a) Normal maan kar chhod de (b) Developmental evaluation ke liye refer kare (c) Walker de (d) Massage bataye

23. Toddler bar bar "nahi" bolta hai aur khud kaam karna chahta hai. Ye kis stage ki baat hai?
(a) Autonomy vs shame & doubt (b) Identity vs role confusion (c) Trust vs mistrust (d) Industry vs inferiority

24. Length measure karna 1 saal ke bachche me –
(a) Tape on wall (b) Standing stadiometer (c) Weight se estimate (d) Lying infantometer

25. Height-for-age < −2 SD kya darshata hai?
(a) Obesity (b) Wasting – acute malnutrition (c) Normal (d) Stunting – chronic malnutrition

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. IAP formula se 5 saal ke bachche ka expected weight –
(a) 14 kg (b) 18 kg (c) 20 kg (d) 25 kg

27. 6 saal ke bachche ki expected height (age × 6 + 77) –
(a) 105 cm (b) 113 cm (c) 120 cm (d) 125 cm

28. Match: P. Trust vs mistrust Q. Initiative vs guilt R. Industry vs inferiority S. Identity vs role confusion — with 1. School age 2. Adolescent 3. Infant 4. Preschool
(a) P-2, Q-4, R-1, S-3 (b) P-4, Q-3, R-1, S-2 (c) P-3, Q-4, R-1, S-2 (d) P-3, Q-1, R-4, S-2

29. Assertion (A): Bachcha baithne se pehle sir sambhalna seekhta hai. Reason (R): Development cephalocaudal direction me hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. 18 mahine ke bachche me milk teeth ki expected sankhya (age − 6) –
(a) 8 (b) 12 (c) 16 (d) 20

Answer Key & Explanation

1-d 5 months.
2-a 1 year.
3-b 75 cm.
4-a 1 year.
5-d 12–18 months.
6-b Lower central incisor ~6 months.
7-b 2½ yrs.
8-d 6–8 weeks.
9-b 3 months.
10-c 8 months.
11-c 9–10 months.
12-c 12–13 months.
13-b 3 years.
14-b 3 years.
15-b Trust vs mistrust.
16-c Sensorimotor (~9 months).
17-a Parallel play.
18-d Anal stage.
19-b 9 months ke normal milestones.
20-d Large, non-toxic, no small parts.

21-a Preschool – dramatic/ associative play.
22-b Not walking by 18 months – red flag.
23-a Autonomy vs shame & doubt.
24-d < 2 yrs lying (infantometer).
25-d Stunting – chronic.
26-b 5 × 2 + 8 = 18 kg.
27-b 6 × 6 + 77 = 113 cm.
28-c Sahi.
29-a Dono sahi, R reason hai.
30-b 18 − 6 = 12.

3 Nutrition, IYCF & Malnutrition

Level 1 – Direct

1. Exclusive breastfeeding kitne mahine tak?
   (a) 3 (b) 4 (c) 6 (d) 12
2. Breast milk ka main carbohydrate –
   (a) Sucrose (b) Glucose (c) Lactose (d) Maltose
3. Breast milk me whey : casein ratio –
   (a) 20 : 80 (b) 40 : 60 (c) 60 : 40 (d) 80 : 20
4. SAM ka MUAC cut-off (6–59 months) –
   (a) < 10 cm (b) < 11.5 cm (c) < 12.5 cm (d) < 13.5 cm
5. Kwashiorkor ka hallmark –
   (a) Severe wasting (b) Old man face (c) Good appetite (d) Oedema
6. "Old man / monkey face" –
   (a) Rickets (b) Scurvy (c) Marasmus (d) Kwashiorkor
7. SAM ke stabilisation phase me kaunsa feed?
   (a) Cow milk (b) RUTF only (c) F-100 (d) F-75
8. SAM me dehydration ke liye –
   (a) Standard ORS (b) ReSoMal (c) Plain water (d) IV NS always
9. Vitamin A deficiency ka earliest symptom –
   (a) Keratomalacia (b) Bitot's spots (c) Night blindness (d) Corneal ulcer
10. Rickets ka earliest clinical sign –
    (a) Harrison's sulcus (b) Bow legs (c) Rachitic rosary (d) Craniotabes
11. Bleeding gums aur "frog position" –
    (a) Rickets (b) Scurvy (c) Pellagra (d) Beriberi
12. 3 D's (dermatitis, diarrhoea, dementia) –
    (a) Beriberi (b) Pellagra (c) Rickets (d) Scurvy
13. Vitamin A prophylaxis ka pehla dose –
    (a) Birth – 50,000 IU (b) 6 weeks – 1 lakh IU (c) 1 yr – 2 lakh IU (d) 9 months – 1 lakh IU
14. Children me sabse common nutritional anaemia –
    (a) Aplastic (b) B12 deficiency (c) Iron deficiency (d) Folate deficiency
15. Anaemia Mukt Bharat – 6–59 months IFA –
    (a) Daily tablet (b) Blue tablet weekly (c) Pink tablet weekly (d) 1 ml syrup biweekly
16. Consumer level par iodised salt me iodine –
    (a) 5 ppm (b) 15 ppm (c) 30 ppm (d) 50 ppm
17. Colostrum me sabse zyada –
    (a) IgA & protein (b) Iron (c) Lactose (d) Fat
18. Wasting kis indicator se pata chalta hai?
    (a) Head circumference (b) Weight-for-height (c) Weight-for-age (d) Height-for-age

Level 2 – Priority & Application

19. SAM bachcha NRC me aaya, temperature 35 °C aur blood glucose 50 mg/dl. Pehle kya?
(a) Discharge (b) Iron syrup (c) Hypoglycaemia & hypothermia ka treatment (d) F-100

20. SAM me iron kab shuru karte hain?
(a) Rehabilitation phase me (b) Kabhi nahi (c) IV first day (d) Admission par

21. 7 mahine ke bachche ke liye complementary feed ki sahi advice –
(a) Gaadha mashed khana, 2–3 baar + breastfeeding (b) Bottle me doodh (c) Sirf fruit juice (d) Patla daal ka paani

22. Mother puchhti hai 4 mahine ke bachche ko paani dena chahiye garmi me. Nurse –
(a) Glucose water (b) Nahi, exclusive breastfeeding me paani ki zarurat nahi (c) Ghutti (d) Haan, 2–3 baar

23. Bachche ko iron syrup dene ki advice –
(a) Khane ke beech, vitamin C ke saath, straw/ dropper se; black stool normal (b) Doodh ke saath (c) Chai ke saath (d) Calcium ke saath

24. Kwashiorkor bachche ki mother ko kis cheez ki kami batayenge?
(a) Vitamin C (b) Fat (c) Protein (d) Iodine

25. MUAC kis haath par measure karte hain?
(a) Right forearm (b) Calf (c) Left mid-upper arm (d) Thigh

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. SAM criteria me kaun NAHI hai?
(a) WFH < −3 SD (b) Height-for-age < −2 SD (c) MUAC < 11.5 cm (d) Bilateral pitting oedema

27. Match: P. Vitamin A Q. Vitamin D R. Vitamin C S. Niacin — with 1. Scurvy 2. Pellagra 3. Bitot's spots 4. Rickets
(a) P-4, Q-3, R-1, S-2 (b) P-2, Q-4, R-1, S-3 (c) P-3, Q-1, R-4, S-2 (d) P-3, Q-4, R-1, S-2

28. Assertion (A): SAM me IV fluids sirf shock me dete hain. Reason (R): SAM bachche me heart failure aur fluid overload ka khatra zyada hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. IAP classification: bachche ka weight expected ka 55 % hai. Grade –
(a) Grade III (b) Grade IV (c) Grade II (d) Grade I

30. Marasmus ke baare me kya SAHI NAHI hai?
(a) Severe wasting (b) Oedema present (c) Alert, hungry (d) Usually < 1 year

Answer Key & Explanation

1-c 6 months.
2-c Lactose.
3-c 60 : 40 (cow milk 20 : 80).
4-b < 11.5 cm.
5-d Oedema.
6-c Marasmus.
7-d F-75.
8-b ReSoMal – low Na, high K.
9-c Night blindness (sign – conjunctival xerosis).
10-d Craniotabes.
11-b Scurvy.
12-b Pellagra.
13-d 9 months – 1 lakh IU.
14-c Iron deficiency.
15-d 1 ml (20 mg iron) biweekly.
16-b 15 ppm.
17-a IgA & protein.
18-b WFH – wasting.
19-c Step 1–2: hypoglycaemia, hypothermia.
20-a Rehabilitation phase.

21-a Thick mashed food 2–3 meals.
22-b EBF – no water.
23-a Vitamin C ↑ absorption; teeth stain se bachav.
24-c Protein.
25-c Left mid-upper arm.
26-b Stunting SAM criterion nahi.
27-d Sahi.
28-a Dono sahi, R reason hai.
29-a 51–60 % = Grade III.
30-b Marasmus me oedema nahi.

4 Immunization

Level 1 – Direct

1. BCG ka route –
   (a) Intramuscular (b) Subcutaneous (c) Oral (d) Intradermal
2. BCG ka dose (1 month se kam age) –
   (a) 0.05 ml (b) 0.1 ml (c) 0.5 ml (d) 1 ml
3. Pentavalent vaccine me kya hota hai?
   (a) DPT + IPV + Hib (b) DPT + BCG (c) DPT + Hep B + Hib (d) MMR + Hep B
4. MR vaccine ka route –
   (a) Intramuscular (b) Subcutaneous (c) Intradermal (d) Oral
5. Hepatitis B birth dose kitne samay ke andar?
   (a) 6 weeks (b) 7 days (c) 15 days (d) 24 hrs
6. Rotavirus vaccine (Rotavac) ka dose –
   (a) 0.1 ml ID (b) 0.5 ml IM (c) 2 drops (d) 5 drops
7. OPV ka dose –
   (a) 0.5 ml (b) 1 drop (c) 2 drops (d) 5 drops
8. fIPV ka route –
   (a) Intradermal (b) Subcutaneous (c) Intramuscular (d) Oral
9. ILR ka temperature –
   (a) −15 to −25 °C (b) 0 to −5 °C (c) +2 to +8 °C (d) +10 to +15 °C
10. Sabse heat-sensitive vaccine –
    (a) Hepatitis B (b) OPV (c) BCG (d) Td
11. Sabse freeze-sensitive vaccine –
    (a) Measles (b) OPV (c) Hepatitis B (d) BCG
12. Vaccine freeze hui ya nahi – kaunsa test?
    (a) VVM (b) Schick test (c) Tuberculin test (d) Shake test
13. Reconstituted MR vaccine kitni der me use karni hai?
    (a) 1 hr (b) 4 hrs (c) 24 hrs (d) 28 days
14. Live attenuated vaccine –
    (a) Hepatitis B (b) Td (c) IPV (d) BCG
15. Toxoid vaccine –
    (a) OPV (b) Rabies (c) Tetanus (d) Measles
16. BCG reconstitution ke liye diluent –
    (a) Ringer lactate (b) Dextrose (c) Sterile water (d) Normal saline
17. Vaccination ke baad observation kitni der?
    (a) 2 hrs (b) 5 min (c) 15 min (d) 30 min
18. MR vaccine ka first dose –
    (a) 6 weeks (b) 9–12 months (c) 14 weeks (d) 16–24 months

Level 2 – Priority & Application

19. Infant ko Pentavalent injection ka site –
(a) Deltoid (b) Anterolateral thigh (c) Gluteal (d) Forearm

20. Bachche ko halka bukhar aur naak beh rahi hai, aaj vaccination day hai. Nurse –
(a) Vaccination postpone (b) Sirf OPV (c) Vaccine de do – minor illness contraindication nahi (d) 1 mahine baad

21. Vaccination ke turant baad bachche ko saans lene me dikkat, urticaria. Pehla drug –
(a) Adrenaline 1:1000 IM (b) Antihistamine oral (c) Hydrocortisone IV (d) Paracetamol

22. VVM ka inner square outer circle se zyada dark hai. Nurse –
(a) Vaccine discard kare (b) Use kare (c) Fridge me rakhe (d) Shake test kare

23. Bachcha OPV lene ke 10 min baad ulti kar deta hai. Kya karein?
(a) Kuch nahi (b) Dose repeat karein (c) Agle mahine (d) IPV de

24. Vaccine carrier me ice packs kaise rakhte hain?
(a) Dry ice (b) Conditioned ice packs (pani ki awaaz aane tak) (c) Bina ice pack (d) Freezer se nikal kar turant

25. Bachcha 6 hafte par vaccine miss kar gaya, ab 4 mahine ka hai. Nurse –
(a) Next year (b) Jahan chhoda wahi se continue, missed doses de (c) Sirf MR (d) Schedule restart

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Open vial policy (28 days) me kaunsa vaccine NAHI aata?
(a) OPV (b) MR (c) Pentavalent (d) Td

27. Match: P. BCG Q. MR R. Pentavalent S. fIPV — with 1. 0.5 ml SC right arm 2. 0.1 ml ID left arm 3. 0.1 ml ID right arm 4. 0.5 ml IM thigh
(a) P-2, Q-1, R-4, S-3 (b) P-3, Q-1, R-4, S-2 (c) P-1, Q-2, R-4, S-3 (d) P-2, Q-4, R-1, S-3

28. Assertion (A): Reconstituted measles vaccine 4 hrs baad discard karte hain. Reason (R): Isme preservative nahi hota, contamination se toxic shock syndrome ho sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Vaccination ka TRUE contraindication kaun sa hai?
(a) Mild diarrhoea (b) Previous dose se anaphylaxis (c) Prematurity (d) Malnutrition

30. Vitamin A ke kitne doses 9 months se 5 saal tak?
(a) 5 (b) 7 (c) 9 (d) 10

Answer Key & Explanation

1-d Intradermal.
2-a 0.05 ml < 1 month; baad me 0.1 ml.
3-c DPT + Hep B + Hib.
4-b Subcutaneous, right upper arm.
5-d Within 24 hrs.
6-d 5 drops.
7-c 2 drops.
8-a Intradermal 0.1 ml.
9-c +2 to +8 °C.
10-b OPV.
11-c Hepatitis B.
12-d Shake test.
13-b 4 hrs.
14-d BCG live.
15-c Tetanus toxoid.
16-d Normal saline.
17-d 30 min.
18-b 9–12 months.
19-b Anterolateral thigh.
20-c Minor illness – false contraindication.
21-a Anaphylaxis – IM adrenaline.
22-a Discard.
23-b 30 min ke andar vomit – repeat.
24-b Conditioned ice packs – freezing se bachav.

25-b Restart nahi, continue.
26-b MR – 4 hrs.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-b Anaphylaxis – true contraindication.
30-c 9 doses.

5 Hospitalised Child, Procedures & Drug Calculations

Level 1 – Direct

1. Hospitalisation me separation anxiety sabse zyada kis age me?
   (a) Adolescent (b) Newborn (c) School age (d) Toddler (6–30 months)
2. Separation anxiety ka pehla stage –
   (a) Detachment (b) Denial (c) Protest (d) Despair
3. Preschool child ka hospital me sabse bada fear –
   (a) Body mutilation / injury (b) Loss of peers (c) Body image (d) Death concept
4. FLACC pain scale kis age ke liye?
   (a) Adults (b) Preterm only (c) 2 months – 7 yrs (d) Adolescents only
5. Wong-Baker FACES scale –
   (a) 1 month (b) ≥ 3 yrs (c) Unconscious (d) Newborn
6. Newborn normal heart rate –
   (a) 60–100 (b) 80–120 (c) 120–160 (d) 160–200
7. Infant me pulse kahan lete hain?
   (a) Radial (b) Carotid (c) Apical (d) Dorsalis pedis
8. Infant me IM injection ka preferred site –
   (a) Deltoid (b) Forearm (c) Vastus lateralis (d) Dorsogluteal
9. 3 saal se kam bachche me ear drops – pinna ko –
   (a) Down & forward (b) Down & back (c) Up & back (d) Up & forward
10. Young's rule –
    (a) Age in months ÷ 150 (b) Weight ÷ 150 × adult dose (c) Age ÷ (Age + 12) × adult dose (d) BSA ÷ 1.73
11. Clark's rule kis par based hai?
    (a) Age (b) Height (c) Weight (d) BSA
12. Paediatric microdrip set me 1 ml =
    (a) 10 drops (b) 15 drops (c) 20 drops (d) 60 drops
13. Cleft lip repair ke baad kaunsa restraint?
    (a) Clove-hitch (b) Mummy (c) Jacket (d) Elbow
14. Venipuncture (scalp/ jugular) ke liye short procedure restraint –
    (a) Ankle (b) Jacket (c) Elbow (d) Mummy restraint
15. Newborn ka heel prick site –
    (a) Centre of heel (b) Lateral/ medial plantar heel (c) Toe (d) Posterior curvature
16. Infant me most reliable urine culture sample –
    (a) Random (b) Suprapubic aspiration (c) Urine bag (d) Diaper
17. Paracetamol ka paediatric dose –
    (a) 5 mg/kg (b) 15 mg/kg/dose (c) 50 mg/kg/dose (d) 100 mg/kg

18. Children me aspirin kyon avoid?
(a) Reye's syndrome (b) Taste (c) Allergy always (d) Cost

? Level 2 – Priority & Application

19. 2 saal ka bachcha admission ke baad chupchap, udaas, khilone me interest nahi. Ye stage –
(a) Despair (b) Normal adjustment (c) Detachment (d) Protest

20. Toddler ki hospital me separation anxiety kam karne ka best tarika –
(a) Sedation (b) Akela chhodna (c) Visiting band (d) Parents ko rooming-in / saath rehne dena

21. Infant ke vital signs lene ka sahi order –
(a) RR → pulse → temperature (b) BP → temperature → RR (c) Temperature → pulse → RR (d) Koi bhi

22. Bachche ko liquid medicine dene ka sahi tareeka –
(a) Oral syringe se cheek ke side me dheere (b) Seedha gale me jaldi (c) Household spoon (d) Bottle ke doodh me mila kar

23. Toddler bed-wetting shuru kar deta hai hospital me (pehle trained tha). Nurse –
(a) Regression samajh kar accept, parents ko reassure (b) Daantna (c) Catheter (d) Psychiatrist

24. Painful procedure kahan karna chahiye?
(a) Child ke bed par (b) Treatment room me (c) Parents ke saamne zabardasti (d) Playroom

25. Restraint lagane ke baad nurse kya check kare?
(a) Circulation har 15–30 min, 2 hr me release (b) Kuch nahi (c) 24 hr baad (d) Sirf shift end par

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. 15 kg bachche ka maintenance fluid (Holliday-Segar) –
(a) 1000 ml/day (b) 1250 ml/day (c) 1500 ml/day (d) 1750 ml/day

27. Paracetamol 15 mg/kg, bachcha 10 kg, syrup 120 mg/5 ml. Kitna ml?
(a) 5.25 ml (b) 6.25 ml (c) 7.5 ml (d) 10 ml

28. Young's rule: 4 saal, adult dose 400 mg. Child dose –
(a) 80 mg (b) 100 mg (c) 133 mg (d) 200 mg

29. Assertion (A): Infant me respiratory rate abdomen dekh kar ginte hain. Reason (R): Infant mainly diaphragmatic (abdominal) breathing karte hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

30. Match: P. Mummy Q. Elbow R. Jacket S. Clove-hitch — with 1. Single limb (IV) 2. Keep in chair/ bed 3. Head-neck procedure 4. Prevent touching face
(a) P-3, Q-4, R-2, S-1 (b) P-3, Q-2, R-4, S-1 (c) P-4, Q-3, R-2, S-1 (d) P-1, Q-4, R-2, S-3

✓ Answer Key & Explanation

1-d Toddler – 6–30 months peak.
2-c Protest.
3-a Body mutilation.
4-c FLACC – 2 months–7 yrs/ non-verbal.
5-b ≥ 3 yrs.
6-c 120–160.
7-c Apical (< 2 yrs), 1 full minute.
8-c Vastus lateralis.
9-b Down & back.
10-c Young's rule.
11-c Weight (lb/150).
12-d 60 drops.
13-d Elbow restraint.
14-d Mummy restraint.

15-b Lateral/ medial plantar heel.
16-b Suprapubic aspiration.
17-b 15 mg/kg/dose.
18-a Reye's syndrome.
19-a Despair.
20-d Rooming-in.
21-a Least invasive first – RR, pulse, temp.
22-a Cheek pocket, slowly – no aspiration.
23-a Regression – temporary, accept.
24-b Treatment room – bed safe place.
25-a Circulation 15–30 min, release 2-hrly.
26-b 1000 + 5 × 50 = 1250 ml.
27-b 150 mg × 5 / 120 = 6.25 ml.
28-b 4/16 × 400 = 100 mg.
29-a Dono sahi, R reason hai.
30-a Sahi.

6 High-Risk Newborn & Neonatal Disorders

? Level 1 – Direct

1. Preterm baby –
(a) < 28 wks (b) < 34 wks (c) < 37 completed weeks (d) < 40 wks

2. VLBW –
(a) < 1000 g (b) < 1500 g (c) < 2000 g (d) < 2500 g

3. Gestational age assessment ka tool –
(a) New Ballard score (b) Kramer (c) Silverman score (d) APGAR

4. Birth ke turant baad heat loss ka main mechanism –
(a) Evaporation (b) Conduction (c) Radiation (d) Convection

5. Newborn me non-shivering thermogenesis –
(a) Muscle shivering (b) Brown fat (c) Sweat (d) Liver glycogen

6. Moderate hypothermia (WHO) –
(a) 37.5 °C (b) 36–36.4 °C (c) < 32 °C (d) 32–35.9 °C

7. KMC ke components –
(a) Incubator only (b) Formula feeding (c) Phototherapy (d) Skin-to-skin, exclusive breastfeeding, early discharge & follow-up

8. Physiological jaundice kab dikhta hai?
(a) First 24 hrs (b) After 24 hrs (c) At birth (d) After 3 weeks

9. Kernicterus brain ke kis part ko affect karta hai?
(a) Frontal lobe (b) Cerebellum (c) Spinal cord (d) Basal ganglia

10. Phototherapy ka effective wavelength –
(a) 250 nm (b) 450–460 nm (blue) (c) 700 nm (d) 900 nm

11. Neonatal sepsis ka earliest sign –
(a) Refusal to feed / lethargy (b) High fever (c) Rash (d) Jaundice

12. Neonatal sepsis ka gold standard test –
(a) Blood culture (b) ESR (c) TLC (d) CRP

13. RDS ka cause –
(a) Surfactant deficiency (b) Infection (c) Meconium (d) Heart defect

14. RDS severity scoring –
(a) Ballard (b) APGAR (c) Bishop (d) Silverman-Anderson

15. Neonatal hypoglycaemia –
(a) < 20 mg/dl (b) < 45 mg/dl (c) < 70 mg/dl (d) < 100 mg/dl

16. NEC ka pathognomonic X-ray sign –
(a) Ground glass (b) Double bubble (c) Pneumatosis intestinalis (d) Steeple sign

17. SNCU kahan hota hai?
(a) Sub-centre (b) PHC (c) Anganwadi (d) District hospital

18. Apnoea of prematurity ka drug –
(a) Adrenaline (b) Atropine (c) Morphine (d) Caffeine citrate

Level 2 – Priority & Application

19. Phototherapy par baby ke liye nurse kya NAHI karegi?
(a) Feeding band (b) Eyes cover (c) Temperature monitor (d) Position 2-hrly change

20. Baby ke paet garam par pair thande. Touch method se –
(a) Cold stress (b) Hyperthermia (c) Hypothermia (d) Normal

21. 32 wks preterm baby ka feeding method –
(a) Direct breastfeeding (b) Bottle (c) Paladai/ katori-spoon ± gavage with EBM (d) NPO 1 week

22. Jaundice pehle 24 ghante me dikhe to –
(a) Pathological – urgent evaluation (b) Breast milk band (c) Ignore (d) Physiological – reassure

23. Hypoglycaemic symptomatic newborn ka treatment –
(a) 25 % dextrose (b) Sugar water oral (c) 10 % dextrose 2 ml/kg IV (d) 50 % dextrose

24. Preterm baby ko oxygen dete waqt target SpO_2 –
(a) 70 % (b) 80 % (c) 90–95 % (d) 100 %

25. NICU me infection rokne ka sabse important tarika –
(a) Visitors band (b) Hand washing (c) Fumigation (d) Antibiotics to all

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Physiological jaundice ka feature NAHI hai –
(a) Disappears by 14 days (b) Peak day 3–5 (c) Conjugated bilirubin > 2 mg/dl (d) After 24 hrs

27. Match: P. Face jaundice Q. Chest R. Legs S. Palms & soles — with 1. ~15 mg/dl 2. ~5 mg/dl 3. ~10 mg/dl 4. > 15 mg/dl
(a) P-2, Q-3, R-1, S-4 (b) P-4, Q-3, R-1, S-2 (c) P-2, Q-1, R-3, S-4 (d) P-3, Q-2, R-1, S-4

28. Assertion (A): Phototherapy me baby ko extra fluid dena padta hai. Reason (R): Phototherapy se insensible water loss badhta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. 3 kg baby ka double-volume exchange transfusion (160 ml/kg) –
(a) 240 ml (b) 360 ml (c) 480 ml (d) 600 ml

30. Preterm baby ki characteristic NAHI hai –
(a) Few sole creases (b) Soft ear with slow recoil (c) Abundant lanugo (d) Fully flexed posture

Answer Key & Explanation

1-c < 37 completed weeks.
2-b < 1500 g.
3-a New Ballard score.
4-a Evaporation (wet baby).
5-b Brown fat.
6-d 32–35.9 °C.
7-d KMC components.
8-b After 24 hrs.
9-d Basal ganglia.
10-b 450–460 nm blue.
11-a Refusal to feed/ lethargy.
12-a Blood culture.
13-a Surfactant deficiency.
14-d Silverman-Anderson.
15-b < 45 mg/dl.
16-c Pneumatosis intestinalis.
17-d District hospital.
18-d Caffeine citrate.
19-a Feeding continue – frequent.
20-a Cold stress.
21-c 32–34 wks – paladai ± gavage.
22-a < 24 hrs – pathological.
23-c 10 % D 2 ml/kg.
24-c 90–95 % – ROP prevention.

25-b Hand hygiene.
26-c Conjugated ↑ – always pathological.
27-a Face 5, chest 10, legs 15, palms > 15.
28-a Dono sahi, R reason hai.
29-c 3 × 160 = 480 ml.
30-d Preterm – extended hypotonic posture.

7 IMNCI, Diarrhoea & Dehydration

Level 1 – Direct

1. India me IMNCI kab shuru hua?
   (a) 1992 (b) 1995 (c) 2003 (d) 2011
2. IMNCI me PINK colour ka matlab –
   (a) Urgent referral (b) No illness (c) Home care (d) OPD treatment
3. IMNCI general danger sign NAHI hai –
   (a) Not able to drink (b) Runny nose (c) Vomits everything (d) Convulsions
4. 2–12 months me fast breathing –
   (a) ≥ 30/min (b) ≥ 40/min (c) ≥ 50/min (d) ≥ 60/min
5. 1–5 saal me fast breathing –
   (a) ≥ 30/min (b) ≥ 40/min (c) ≥ 50/min (d) ≥ 60/min
6. Diarrhoea (WHO) –
   (a) ≥ 1 loose stool (b) ≥ 2 in 12 hrs (c) ≥ 3 loose stools in 24 hrs (d) ≥ 5 formed stools
7. Persistent diarrhoea –
   (a) ≥ 3 days (b) ≥ 7 days (c) ≥ 14 days (d) ≥ 30 days
8. 2 saal se kam bachchon me diarrhoea ka sabse common cause –
   (a) Cholera (b) Shigella (c) Rotavirus (d) Giardia
9. Bachchon me dysentery ka common cause –
   (a) Shigella (b) Cholera (c) Norovirus (d) Rotavirus
10. Low-osmolarity ORS ki osmolarity –
    (a) 200 mOsm/L (b) 245 mOsm/L (c) 311 mOsm/L (d) 330 mOsm/L
11. Low-osmolarity ORS me sodium –
    (a) 45 mmol/L (b) 75 mmol/L (c) 90 mmol/L (d) 110 mmol/L
12. 6 mahine se upar bachche ko zinc ka dose –
    (a) 5 mg × 30 days (b) 10 mg × 7 days (c) 20 mg × 14 days (d) 40 mg × 14 days
13. Some dehydration me Plan B –
    (a) ORS 75 ml/kg in 4 hrs (b) NPO (c) Home fluids only (d) IV RL 100 ml/kg
14. Severe dehydration ka IV fluid –
    (a) 3 % saline (b) Ringer lactate (c) 5 % dextrose (d) Plasma
15. Skin pinch "very slowly" –
    (a) > 1 sec (b) > 2 sec (c) > 5 sec (d) > 10 sec
16. Rice-water stools –
    (a) Rotavirus (b) Shigella (c) Amoebiasis (d) Cholera
17. Raat ko perianal itching –
    (a) Tapeworm (b) Hookworm (c) Pinworm (d) Roundworm
18. Hookworm se kya hota hai?
    (a) Hepatitis (b) Rickets (c) Jaundice (d) Iron-deficiency anaemia

Level 2 – Priority & Application

19. Diarrhoea wale bachche ki maa ko Plan A me kya NAHI batana?
(a) Extra fluids, ORS (b) Zinc 14 days (c) Kab wapas aana (d) Khana band karna

20. Plan B ke dauran bachcha ulti karta hai. Nurse –
(a) Antiemetic inject (b) IV shuru (c) ORS band (d) 10 min ruk kar dheere dheere phir de

21. Bachcha lethargic, sunken eyes, drinks poorly, skin pinch very slow. Classification –
(a) No dehydration (b) Persistent diarrhoea (c) Severe dehydration (d) Some dehydration

22. Acute watery diarrhoea me antibiotics –
(a) Loperamide (b) Routinely (c) Nahi – sirf dysentery/ cholera jaise cases me (d) Hamesha metronidazole

23. ORS banane ki sahi advice –
(a) Ubal kar (b) 1 packet ko 1 litre saaf paani me, 24 hrs me use (c) Cheeni mila kar (d) Aadha litre me

24. Chest indrawing kaise dekhte hain?
(a) Saans andar lene par lower chest wall andar jana (b) Neck muscle (c) Nostril (d) Bahar nikalna

25. Pinworm ke liye sabse achha diagnostic test –
(a) Stool routine (b) Blood test (c) X-ray (d) Cellophane (Scotch) tape test – subah

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. 10 kg bachche ko Plan B me 4 hrs me ORS –
(a) 300 ml (b) 500 ml (c) 750 ml (d) 1000 ml

27. 8 mahine ka bachcha, 7 kg, severe dehydration. Pehle 1 ghante me RL –
(a) 70 ml (b) 210 ml (c) 490 ml (d) 700 ml

28. Assertion (A): ORS me glucose hota hai. Reason (R): Glucose intestine me sodium absorption ko badhata hai (co-transport).
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Match: P. No dehydration Q. Some R. Severe S. Dysentery — with 1. Ciprofloxacin 2. ORS 75 ml/kg 3. Plan A home 4. IV RL 100 ml/kg
(a) P-3, Q-2, R-4, S-1 (b) P-1, Q-2, R-4, S-3 (c) P-2, Q-3, R-4, S-1 (d) P-3, Q-4, R-2, S-1

30. Plan C me 2 saal ke bachche ko first 30 ml/kg kitne time me?
(a) 1 hr (b) 2½ hrs (c) 5 hrs (d) 30 min

Answer Key & Explanation

1-c 2003.
2-a Urgent referral.
3-b Runny nose danger sign nahi.
4-c ≥ 50/min.
5-b ≥ 40/min.
6-c ≥ 3 loose stools/24 hrs.
7-c ≥ 14 days.
8-c Rotavirus.
9-a Shigella.
10-b 245 mOsm/L.
11-b 75 mmol/L.
12-c 20 mg × 14 days.
13-a 75 ml/kg in 4 hrs.
14-b Ringer lactate.
15-b > 2 sec.
16-d Cholera.
17-c Pinworm (Enterobius).
18-d Iron-deficiency anaemia.
19-d Feeding continue karna hai.
20-d Wait 10 min, then slowly.
21-c Severe dehydration.
22-c Routine antibiotic nahi.
23-b 1 packet/1 litre, 24 hrs.
24-a Lower chest wall in on inspiration.
25-d Tape test.
26-c 75 × 10 = 750 ml.

27-b 30 × 7 = 210 ml in 1 hr (infant).
28-a Dono sahi, R reason hai.
29-a Sahi.
30-d ≥ 12 months – 30 min.

8 Respiratory Disorders

Level 1 – Direct

1. 8 saal se kam bachche me airway ka narrowest part –
   (a) Bronchus (b) Cricoid cartilage (c) Vocal cords (d) Trachea
2. Croup ka sabse common cause –
   (a) Adenovirus (b) Parainfluenza virus (c) Hib (d) RSV
3. Croup ka X-ray sign –
   (a) Snowstorm (b) Steeple sign (c) Thumb sign (d) Boot shape
4. Acute epiglottitis ka organism –
   (a) Parainfluenza (b) Strep pyogenes (c) RSV (d) H. influenzae type b
5. Epiglottitis ka X-ray sign –
   (a) Steeple sign (b) Double bubble (c) Air bronchogram (d) Thumb sign
6. Bronchiolitis ka sabse common cause –
   (a) Pneumococcus (b) Staph (c) Mycoplasma (d) RSV
7. Bachchon me bacterial pneumonia ka common cause –
   (a) Pseudomonas (b) Streptococcus pneumoniae (c) E. coli (d) Klebsiella
8. Pneumonia ka most sensitive sign –
   (a) Cyanosis (b) Fever (c) Fast breathing (d) Cough
9. Otitis media bachchon me zyada kyon?
   (a) Narrow nose (b) Small tonsils (c) Long ear canal (d) Short, wide, horizontal Eustachian tube
10. Foreign body zyada kis bronchus me jata hai?
    (a) Left (b) Right (c) Dono barabar (d) Trachea only
11. Cystic fibrosis ka diagnostic test –
    (a) Chest X-ray (b) Spirometry (c) Sweat chloride test (d) Mantoux
12. Cystic fibrosis ka earliest manifestation (newborn) –
    (a) Diabetes (b) Haemoptysis (c) Meconium ileus (d) Clubbing
13. Asthma me reliever drug –
    (a) Budesonide (b) Montelukast (c) Theophylline SR (d) Salbutamol
14. Asthma ka most effective long-term controller –
    (a) Inhaled corticosteroid (b) Antibiotic (c) Antihistamine (d) Oral salbutamol
15. Mantoux test positive –
    (a) ≥ 10 mm induration (b) ≥ 5 mm erythema (c) ≥ 20 mm erythema (d) Any redness
16. Chhote bachche me TB ke liye sample –
    (a) Gastric aspirate (subah) (b) Stool (c) CSF always (d) Urine
17. Tonsillectomy ke baad bleeding ka earliest sign –
    (a) Hypotension (b) Frequent swallowing (c) Fever (d) Vomiting dark blood after 3 days
18. Laryngomalacia kis cheez ka common cause hai?
    (a) Wheeze (b) Congenital stridor (c) Pneumonia (d) Asthma

Level 2 – Priority & Application

19. 4 saal ka bachcha drooling, high fever, tripod position, toxic. Nurse kya NAHI karegi?
(a) Emergency airway equipment ready (b) Child ko calm rakhna (c) Parent ki god me baithne dena (d) Throat examination tongue depressor se

20. Tonsillectomy ke baad position –
(a) Prone / side-lying (b) Trendelenburg (c) Fowler's 90° (d) Supine

21. Tonsillectomy ke baad kaunsa food avoid karein?
(a) Red coloured / citrus juice (b) Cold water (c) Jelly (d) Ice cream

22. Bronchiolitis infant ka mainstay treatment –
(a) Surgery (b) Supportive – humidified O_2, suction, hydration (c) Steroids routinely (d) Antibiotics

23. 1 saal ka bachcha choke kar raha hai, cough nahi kar pa raha. First aid –
(a) Water pilana (b) Abdominal thrusts (c) Blind finger sweep (d) 5 back blows + 5 chest thrusts

24. Pneumonia wale bachche ki position –
(a) Semi-Fowler's / propped up (b) Trendelenburg (c) Flat supine (d) Prone

25. 3 saal ke asthmatic bachche ke liye inhaler device –
(a) Nebuliser only lifelong (b) DPI (c) Oral tablets only (d) MDI + spacer with face mask

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Croup vs epiglottitis – epiglottitis ka feature NAHI hai –
(a) Sudden onset (b) High fever (c) Drooling (d) Barking cough

27. Match: P. Croup Q. Epiglottitis R. Bronchiolitis S. Cystic fibrosis — with 1. RSV 2. Sweat chloride 3. Steeple sign 4. Thumb sign
(a) P-4, Q-3, R-1, S-2 (b) P-3, Q-1, R-4, S-2 (c) P-2, Q-4, R-1, S-3 (d) P-3, Q-4, R-1, S-2

28. Assertion (A): Foreign body right main bronchus me zyada jata hai. Reason (R): Right bronchus chauda, chhota aur zyada vertical hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Asthma attack me "silent chest" ka matlab –
(a) Severe obstruction – ominous sign (b) Pneumonia (c) Improvement (d) Normal

30. Cystic fibrosis diet me kya SAHI NAHI hai?
(a) Fat restricted, low calorie (b) High calorie (c) Pancreatic enzymes with meals (d) Extra salt

Answer Key & Explanation

1-b Cricoid cartilage.
2-b Parainfluenza.
3-b Steeple sign.
4-d Hib.
5-d Thumb sign.
6-d RSV.
7-b Pneumococcus.
8-c Fast breathing.
9-d Short horizontal Eustachian tube.
10-b Right – wider, vertical.
11-c Sweat chloride > 60.
12-c Meconium ileus.
13-d Salbutamol (SABA).
14-a ICS.
15-a ≥ 10 mm induration.
16-a Gastric aspirate.
17-b Frequent swallowing.
18-b Congenital stridor.
19-d Throat exam – laryngospasm risk.
20-a Prone/ side – drainage.
21-a Red (blood jaisa dikhe), citrus (irritation).
22-b Supportive.
23-d Infant – back blows + chest thrusts.
24-a Propped up.

25-d < 4 yrs – MDI + spacer + mask.
26-d Barking cough – croup.
27-d Sahi.
28-a Dono sahi, R reason hai.
29-a Very little air movement – ominous.
30-a CF – high calorie, high fat with enzymes.

9 Cardiovascular Disorders

Level 1 – Direct

1. Sabse common congenital heart disease –
   (a) VSD (b) TOF (c) PDA (d) ASD
2. Infancy ke baad sabse common cyanotic CHD –
   (a) Tricuspid atresia (b) Truncus arteriosus (c) TGA (d) TOF
3. Newborn me sabse common cyanotic CHD –
   (a) TOF (b) TGA (c) ASD (d) VSD
4. Boot-shaped heart –
   (a) TAPVC (b) ASD (c) TGA (d) TOF
5. Egg on string appearance –
   (a) TGA (b) PDA (c) TOF (d) Coarctation
6. Continuous machinery murmur –
   (a) ASD (b) TOF (c) VSD (d) PDA
7. Wide fixed split S2 –
   (a) Coarctation (b) VSD (c) ASD (d) PDA
8. Radio-femoral delay aur rib notching –
   (a) Coarctation of aorta (b) Aortic regurgitation (c) ASD (d) TOF
9. Preterm me PDA band karne ki dawa –
   (a) Indomethacin / ibuprofen (b) Digoxin (c) Adrenaline (d) Prostaglandin E1
10. Ductus ko khula rakhne ki dawa –
    (a) Propranolol (b) Prostaglandin E1 (c) Indomethacin (d) Paracetamol
11. Tet spell me position –
    (a) Prone (b) Knee-chest (c) Trendelenburg (d) Supine
12. TOF me kaunsa defect NAHI hota?
    (a) ASD (b) VSD (c) Pulmonary stenosis (d) Overriding aorta
13. Rheumatic fever kis infection ke baad hota hai?
    (a) Group A strep pharyngitis (b) Viral fever (c) Staph skin infection (d) TB
14. Rheumatic fever me antibody test –
    (a) VDRL (b) ANA (c) Widal (d) ASO titre
15. Rheumatic fever secondary prophylaxis (India) –
    (a) Amoxicillin daily (b) Steroids (c) Benzathine penicillin 1.2 MU every 21 days (d) Aspirin
16. Kawasaki disease ki serious complication –
    (a) Coronary artery aneurysm (b) Renal failure (c) Stroke (d) Hepatitis
17. Infant me CHF ka most reliable sign –
    (a) Raised JVP (b) Hepatomegaly (c) Ascites (d) Pedal oedema
18. Down syndrome me characteristic heart defect –
    (a) Coarctation (b) Ebstein (c) AVSD (endocardial cushion defect) (d) TGA

Level 2 – Priority & Application

19. 1 saal ke infant ko digoxin dena hai, apical pulse 84/min. Nurse –
(a) Double dose (b) Dose withhold karke doctor ko inform (c) Half dose (d) Dose de de

20. Digoxin dene se pehle pulse kaise check karein?
(a) Carotid 30 sec (b) Brachial 10 sec (c) Radial 15 sec (d) Apical 1 full minute

21. TOF bachcha rote hue achanak zyada blue aur limp ho gaya. Pehla nursing action –
(a) Flat supine (b) Knee-chest position, calm karna, O_2 (c) Walk karana (d) Feeding

22. Infant feeding karte waqt pasina aata hai, jaldi thak jata hai, weight nahi badhta. Suspect –
(a) Normal (b) Hypothyroid (c) Pneumonia (d) CHF

23. CHF infant ko feed kaise karein?
(a) Lying flat (b) Feeding band (c) Ek baar bada feed (d) Small frequent feeds, semi-upright, soft nipple

24. Cardiac catheterisation ke baad nurse sabse pehle kya dekhe?
(a) Play (b) Bath (c) Diet (d) Insertion site ke neeche distal pulse, colour, warmth

25. Rheumatic fever wale bachche me carditis par –
(a) School (b) Exercise (c) Bed rest (d) Football

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Jones major criteria me NAHI hai –
(a) Fever (b) Sydenham's chorea (c) Polyarthritis (d) Carditis

27. Match: P. TOF Q. TGA R. TAPVC S. PDA — with 1. Machinery murmur 2. Snowman 3. Egg on string 4. Boot shape
(a) P-3, Q-4, R-2, S-1 (b) P-1, Q-3, R-2, S-4 (c) P-4, Q-3, R-2, S-1 (d) P-4, Q-2, R-3, S-1

28. Assertion (A): TOF bachche squat karte hain. Reason (R): Squatting se systemic vascular resistance badhta hai aur right-to-left shunt kam hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Acyanotic CHD NAHI hai –
(a) VSD (b) PDA (c) ASD (d) Tricuspid atresia

30. Digoxin toxicity ka bachchon me earliest sign –
(a) Vomiting / nausea (b) Tachycardia (c) Fever (d) Rash

Answer Key & Explanation

1-a VSD.
2-d TOF.
3-b TGA.
4-d TOF.
5-a TGA.
6-d PDA.
7-c ASD.
8-a Coarctation.
9-a Indomethacin/ ibuprofen.
10-b PGE1.
11-b Knee-chest.
12-a ASD TOF ka part nahi (pentalogy me).
13-a GAS pharyngitis.
14-d ASO titre.
15-c Every 21 days.
16-a Coronary aneurysm.
17-b Hepatomegaly.
18-c AVSD.
19-b Infant < 90–110 – withhold.
20-d Apical 1 min.
21-b Tet spell management.
22-d CHF – feeding sweating.
23-d Small frequent feeds.
24-d Distal circulation.
25-c Bed rest.
26-a Fever – minor criterion.

27-c Sahi.
28-a Dono sahi, R reason hai.
29-d Tricuspid atresia – cyanotic.
30-a Vomiting/ nausea; bradycardia.

10 Gastrointestinal Disorders

Level 1 – Direct

1. Cleft lip repair ki usual age –
   (a) 5 years (b) ~3 months (rule of 10s) (c) 2 years (d) Birth
2. Cleft palate repair –
   (a) 10 years (b) 5 years (c) Birth (d) 9–18 months
3. Cleft lip suture line ko protect karne wala device –
   (a) Pavlik harness (b) Splint (c) Spica (d) Logan bow
4. Sabse common TEF type –
   (a) H-type (b) Type C (c) Type B (d) Type A
5. TEF ka pehla sign (newborn) –
   (a) Excessive frothy saliva / drooling (b) Jaundice (c) Projectile vomiting (d) Diarrhoea
6. Pyloric stenosis ki age –
   (a) 6 months (b) 2–6 weeks (c) 2 years (d) Birth
7. Pyloric stenosis me vomiting –
   (a) Bloody (b) Projectile non-bilious (c) Bilious (d) Faecal
8. Pyloric stenosis ki surgery –
   (a) Kasai (b) Ponseti (c) Ramstedt pyloromyotomy (d) Swenson
9. Red currant jelly stool –
   (a) Pyloric stenosis (b) Intussusception (c) Coeliac (d) Hirschsprung
10. Intussusception ka USG sign –
    (a) Steeple (b) Target / doughnut sign (c) String sign (d) Double bubble
11. Hirschsprung disease me kya absent hota hai?
    (a) Bile ducts (b) Anal opening (c) Ganglion cells (d) Villi
12. Hirschsprung ka confirmatory test –
    (a) Stool test (b) Barium meal (c) USG (d) Rectal biopsy
13. Hirschsprung me newborn ka early sign –
    (a) Red stool (b) Jaundice (c) Meconium 48 hrs tak pass na hona (d) Projectile vomiting
14. Omphalocele –
    (a) Diaphragm defect (b) Hernia groin (c) Bina sac, right side (d) Sac se covered, umbilicus se
15. Coeliac disease me kya avoid karein?
    (a) Maize (b) Milk always (c) Wheat, barley, rye (gluten) (d) Rice
16. Biliary atresia ki surgery (Kasai) kab tak best?
    (a) 1 year (b) 60 days se pehle (c) Kabhi bhi (d) 2 years
17. Meckel's diverticulum – "rule of 2s" me symptom –
    (a) Jaundice (b) Vomiting (c) Painless rectal bleeding (d) Stridor
18. Umbilical hernia –
    (a) Mostly 3–5 saal tak apne aap band (b) Coin se strapping (c) Turant surgery (d) Hamesha incarcerate

Level 2 – Priority & Application

19. Cleft lip repair ke baad baby ki position –
(a) Prone (b) Sitting 90° (c) Trendelenburg (d) Supine / side-lying

20. Cleft palate repair ke baad position –
(a) Supine (b) Knee-chest (c) Trendelenburg (d) Prone / side-lying

21. TEF suspected newborn – nurse ka immediate action –
(a) Feed dena (b) Discharge (c) Bottle feed (d) NPO, head-up, upper pouch ki continuous suction

22. Intussusception ke baad hydrostatic reduction, bachcha normal brown stool pass karta hai. Matlab –
(a) Perforation (b) Complication (c) Reduction successful – doctor ko report (d) Normal ignore

23. Hirschsprung bachche ke liye washout fluid –
(a) Hypertonic phosphate (b) Tap water (c) Normal saline (d) Soap water

24. Gastroschisis newborn ki first care –
(a) Sterile warm moist saline dressing + plastic wrap (b) Feed (c) Bowel ko andar push (d) Bowel ko dry chhodna

25. Pyloric stenosis bachche me surgery se pehle priority –
(a) Turant surgery (b) Feeding (c) Fluid aur electrolyte correction (d) Antibiotic

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Pyloric stenosis me metabolic picture –
(a) Hyperkalaemia (b) Hypochloraemic hypokalaemic metabolic alkalosis (c) Respiratory acidosis (d) Hyperchloraemic acidosis

27. Match: P. Pyloric stenosis Q. Intussusception R. Hirschsprung S. Biliary atresia — with 1. Kasai 2. Olive mass 3. Ribbon stools 4. Sausage mass
(a) P-1, Q-4, R-3, S-2 (b) P-2, Q-4, R-3, S-1 (c) P-2, Q-3, R-4, S-1 (d) P-4, Q-2, R-3, S-1

28. Assertion (A): TEF baby ko head-up position me rakhte hain. Reason (R): Isse gastric acid fistula se lungs me reflux nahi hota.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Rule of 10s (cleft lip) me NAHI hai –
(a) 10 lb weight (b) 10 teeth (c) Hb 10 g/dl (d) 10 weeks age

30. Omphalocele vs gastroschisis – SAHI kaunsa?
(a) Gastroschisis me sac (b) Omphalocele me sac, associated anomalies zyada (c) Dono same (d) Omphalocele right of umbilicus

Answer Key & Explanation

1-b ~3 months.
2-d 9–18 months.
3-d Logan bow.
4-b Type C ~85 %.
5-a Frothy saliva.
6-b 2–6 weeks.
7-b Projectile non-bilious.
8-c Ramstedt.
9-b Intussusception.
10-b Target/ doughnut.
11-c Ganglion cells.
12-d Rectal biopsy.
13-c Delayed meconium.
14-d Sac covered.
15-c Gluten.
16-b < 60 days.

17-c Painless bleeding.
18-a Spontaneous closure.
19-d Supine/ side – lip par pressure nahi.
20-d Prone/ side – drainage.
21-d NPO, head-up, suction.
22-c Reduction successful.
23-c NS – tap water se water intoxication.
24-a Moist sterile cover.
25-c Not surgical emergency – stabilise first.
26-b HCl loss – alkalosis.
27-b Sahi.
28-a Dono sahi, R reason hai.
29-b WBC < 10,000 hota hai, teeth nahi.
30-b Omphalocele – sac + anomalies.

11 Genitourinary & Renal Disorders

Level 1 – Direct

1. Bachchon me nephrotic syndrome ka sabse common type –
   (a) FSGS (b) Minimal change disease (c) Membranous (d) PSGN
2. Nephrotic syndrome me serum albumin –
   (a) < 2.5 g/dl (b) > 4 g/dl (c) > 5 g/dl (d) Normal
3. Nephrotic syndrome me oedema pehle kahan dikhta hai?
   (a) Feet (b) Abdomen (c) Periorbital (subah) (d) Scrotum
4. Nephrotic syndrome initial treatment –
   (a) Prednisolone 2 mg/kg/day × 6 wks (b) Insulin (c) Penicillin (d) Dialysis
5. AGN me urine ka colour –
   (a) Cola / smoky coloured (b) Frothy yellow (c) Green (d) Milky
6. AGN (PSGN) me C3 complement –
   (a) Normal (b) High (c) Absent always (d) Low
7. AGN kis infection ke baad?
   (a) E. coli (b) TB (c) Group A strep (throat/ skin) (d) Virus
8. Bachchon me UTI ka sabse common organism –
   (a) Pseudomonas (b) E. coli (c) Candida (d) Staph
9. UTI ka gold standard –
   (a) Urine routine (b) USG (c) CBC (d) Urine culture
10. Wilms tumour kis organ ka hai?
    (a) Liver (b) Adrenal (c) Bladder (d) Kidney
11. Hypospadias me urethral opening –
    (a) Normal (b) Dorsal side (c) Perineum always (d) Ventral (under) side
12. Undescended testis ke liye orchidopexy –
    (a) Never (b) 5 yrs (c) Puberty (d) 6–12 months
13. Enuresis kis age ke baad diagnose hoti hai?
    (a) 2 yrs (b) 3 yrs (c) 5 yrs (d) 10 yrs
14. Enuresis ka sabse effective long-term treatment –
    (a) Antibiotics (b) Fluid restriction full day (c) Punishment (d) Enuresis alarm
15. HUS triad me NAHI hai –
    (a) Haemolytic anaemia (b) AKI (c) Thrombocytopenia (d) Hypertension crisis always
16. Normal child urine output –
    (a) 0.5 ml/kg/hr (b) 1 ml/kg/hr (c) 5 ml/kg/hr (d) 10 ml/kg/hr
17. Testicular torsion ka sign –
    (a) Transillumination positive (b) Absent cremasteric reflex, sudden pain (c) Painless swelling (d) Fever only
18. Phimosis physiological hoti hai –
    (a) 15 saal tak (b) Kabhi nahi (c) Sirf newborn (d) 3–5 saal tak

Level 2 – Priority & Application

19. Nephrotic bachche ke fluid status ka best indicator –
(a) BP only (b) Urine colour (c) Daily weight (d) Thirst

20. Nephrotic syndrome diet –
(a) High protein, high salt (b) No added salt, normal protein (c) Protein free (d) Fluid unlimited

21. Wilms tumour bachche ke liye nursing order –
(a) Abdomen roz palpate (b) Abdominal massage (c) Hot pack (d) "Do not palpate abdomen"

22. Hypospadias wale baby ke parents ko kya advice?
(a) Turant circumcision (b) Kuch nahi (c) Circumcision mat karwana – foreskin repair me lagti hai (d) Puberty tak wait

23. AGN me fluid allowance –
(a) Insensible loss + pichhle din ka urine output (b) 3 L/day (c) Zero (d) Unlimited

24. Girls ko UTI se bachne ki advice –
(a) Fluids kam (b) Urine rok kar rakho (c) Front to back wipe, cotton underwear, bubble bath avoid (d) Back to front wipe

25. 7 saal ka bachcha raat me bistar geela karta hai, parents daantte hain. Nurse ki advice –
(a) Reassure, star chart, sone se pehle urine, alarm (b) Diaper lifelong (c) Fluid bilkul band (d) Punishment jaari rakho

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Nephrotic vs nephritic – nephrotic ka feature NAHI hai –
(a) Gross haematuria + hypertension (b) Massive proteinuria (c) Hypoalbuminaemia (d) Hyperlipidaemia

27. Match: P. Nephrotic Q. AGN R. Wilms S. HUS — with 1. Don't palpate 2. Low C3 3. Hyperlipidaemia 4. E. coli O157 dysentery
(a) P-3, Q-1, R-2, S-4 (b) P-3, Q-2, R-1, S-4 (c) P-4, Q-2, R-1, S-3 (d) P-2, Q-3, R-1, S-4

28. Assertion (A): Nephrotic syndrome me infection ka khatra zyada hota hai. Reason (R): Immunoglobulin urine me lose hota hai aur steroids immunity kam karte hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. 4 saal ke bachche ki bladder capacity [(age + 2) × 30] –
(a) 120 ml (b) 150 ml (c) 180 ml (d) 240 ml

30. Steroid therapy ka side effect NAHI hai –
(a) Hypertension (b) Hyperglycaemia (c) Cushingoid face (d) Weight loss & ↓ appetite

Answer Key & Explanation

1-b Minimal change.
2-a < 2.5 g/dl.
3-c Periorbital, morning.
4-a 2 mg/kg/day × 6 wks.
5-a Cola coloured.
6-d Low C3.
7-c Group A strep.
8-b E. coli.
9-d Culture.
10-d Kidney.
11-d Ventral.
12-d 6–12 months.
13-c 5 yrs.
14-d Alarm.
15-d HUS triad – MAHA, thrombocytopenia, AKI.
16-b 1 ml/kg/hr.
17-b Torsion – emergency.
18-d 3–5 yrs.

19-c Daily weight.
20-b No added salt, normal protein.
21-d Don't palpate – seeding.
22-c No circumcision.
23-a Insensible + urine output.
24-c Front to back.
25-a Positive approach.
26-a Gross haematuria + HTN – nephritic.
27-b Sahi.
28-a Dono sahi, R reason hai.
29-c (4 + 2) × 30 = 180 ml.
30-d Steroid – weight gain, appetite ↑.

12 Neurological Disorders

Level 1 – Direct

1. Infant me hydrocephalus ka earliest sign –
(a) Headache (b) Diplopia (c) Papilloedema (d) Head circumference ka tezi se badhna

2. "Setting-sun" eyes –
(a) Hydrocephalus (b) Hypothyroidism (c) Down syndrome (d) Meningitis

3. Macewen's (cracked-pot) sign –
(a) Rickets (b) Hydrocephalus (c) Scurvy (d) CP

4. Hydrocephalus ka common surgical treatment –
(a) Laminectomy (b) VP shunt (c) Lumbar puncture daily (d) Craniotomy always

5. Sabse severe spina bifida type –
(a) Occulta (b) Meningomyelocele (c) Meningocele (d) Lipoma

6. Neural tube defect ka antenatal marker –
(a) ↑ AFP (b) ↑ TSH (c) ↓ hCG (d) ↓ PAPP-A

7. Meningitis me Kernig's sign –
(a) Neck flex par hip flex (b) Hip flex karke knee extend karne par pain (c) Pupil dilatation (d) Babinski

8. Infant me meningitis ka important sign –
(a) Bulging fontanelle (b) Neck rigidity hamesha (c) Sunken fontanelle (d) Kernig's always

9. Bacterial meningitis me CSF glucose –
(a) Absent always (b) Normal (c) Increased (d) Decreased

10. Meningitis ki sabse common sequela –
(a) Diabetes (b) Blindness (c) Sensorineural deafness (d) Paralysis

11. Febrile seizure ki age –
(a) 5–15 yrs (b) Birth – 1 month (c) Adolescents (d) 6 months – 5 yrs

12. Simple febrile seizure –
(a) Recurrent same day (b) > 30 min (c) Focal (d) < 15 min, generalised, 24 hrs me ek baar

13. Status epilepticus –
(a) Seizure ≥ 5 min (b) 2 seizures in a day (c) ≥ 1 hr (d) ≥ 1 min

14. Status epilepticus me first-line drug –
(a) Carbamazepine (b) Benzodiazepine (lorazepam/ midazolam) (c) Valproate oral (d) Phenytoin

15. Phenytoin ka common side effect –
(a) Hair loss (b) Gum hypertrophy (c) Weight loss (d) Bradycardia only

16. Cerebral palsy ka sabse common type –
(a) Ataxic (b) Spastic (c) Mixed (d) Athetoid

17. Reye's syndrome kis drug se?
(a) Paracetamol (b) Ibuprofen (c) Amoxicillin (d) Aspirin

18. Gowers' sign –
(a) CP (b) Duchenne muscular dystrophy (c) GBS (d) Polio

Level 2 – Priority & Application

19. Meningomyelocele newborn ki pre-op position –
(a) Sitting (b) Prone (c) Side with sac pressed (d) Supine

20. Sac par dressing –
(a) Dry gauze tight (b) Povidone thick paste (c) Sterile moist normal saline dressing (d) Open, no dressing

21. VP shunt ke baad bachche ki position –
(a) Head-low (b) Prone on shunt (c) Flat, unoperated side par (d) Operated side par

22. Febrile seizure ke dauran parents ko first aid advice –
(a) Side me litana, kuch muh me na dalna, time note (b) Paani pilana (c) Kas kar pakadna (d) Muh me chammach

23. Meningococcal meningitis me isolation –
(a) Droplet – antibiotic ke 24 hrs baad tak (b) Airborne lifelong (c) 2 weeks (d) Nahi chahiye

24. Phenytoin IV kis fluid me dilute karein?
(a) Normal saline (b) 5 % dextrose (c) RL (d) Sterile water

25. CP bachche ko feeding karate waqt –
(a) Fast feeding (b) Upright, chin tuck, small bites – aspiration prevention (c) Only liquids lying down (d) Lying flat

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Hydrocephalus ka sign NAHI hai –
(a) Separated sutures (b) High-pitched cry (c) Bulging fontanelle (d) Sunken fontanelle

27. Match: P. Absence seizure Q. Infantile spasms R. Neonatal seizures S. Febrile seizure — with 1. Phenobarbitone first-line 2. Ethosuximide 3. Paracetamol + first aid, no long-term AED 4. ACTH/ vigabatrin
(a) P-3, Q-4, R-1, S-2 (b) P-2, Q-4, R-1, S-3 (c) P-2, Q-1, R-4, S-3 (d) P-4, Q-2, R-1, S-3

28. Assertion (A): Folic acid preconception lene se NTD kam hote hain. Reason (R): Neural tube day 28 tak band ho jati hai, jab aksar mother ko pregnancy pata bhi nahi hoti.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Bacterial meningitis CSF me kya NAHI milega?
(a) ↑ Protein (b) Turbid (c) ↑ Glucose (d) ↑ Neutrophils

30. VP shunt ki sabse common overall complication –
(a) Shunt malfunction / blockage (b) Allergy (c) Hypotension (d) Fracture

Answer Key & Explanation

1-d Rapid ↑ OFC.
2-a Hydrocephalus.
3-b Hydrocephalus.
4-b VP shunt.
5-b Meningomyelocele.
6-a ↑ AFP.
7-b Kernig's.
8-a Bulging fontanelle.
9-d Decreased.
10-c Deafness.
11-d 6 months – 5 yrs.
12-d Simple criteria.
13-a ≥ 5 min.
14-b Benzodiazepine.
15-b Gum hypertrophy.
16-b Spastic.
17-d Aspirin.
18-b DMD.

19-b Prone.
20-c Moist saline.
21-c Flat, unoperated side.
22-a Recovery position.
23-a Droplet 24 hrs.
24-a NS – dextrose me precipitate.
25-b Upright, chin tuck.
26-d Sunken – dehydration.
27-b Sahi.
28-a Dono sahi, R reason hai.
29-c Glucose ↓ hota hai.
30-a Malfunction.

13 Haematology & Oncology

Level 1 – Direct

1. Physiological anaemia of infancy kab hota hai (term)?
   (a) 5 years (b) Birth (c) 2–3 months (d) 1 year
2. Thalassaemia major ke symptoms kab shuru hote hain?
   (a) ~6 months (b) 5 years (c) Puberty (d) Birth
3. Thalassaemia me skull X-ray –
   (a) Normal (b) Copper-beaten (c) Hair-on-end (d) Pepper-pot
4. Thalassaemia me Hb electrophoresis –
   (a) ↑ HbC (b) Normal (c) ↑ HbS (d) ↑ HbF
5. Desferrioxamine kis liye?
   (a) Anaemia treatment (b) Iron overload chelation (c) Pain (d) Infection
6. Sickle cell disease ka earliest presentation infants me –
   (a) Gallstones (b) Stroke (c) Hand-foot syndrome (dactylitis) (d) Leg ulcer
7. Sickle cell long-term drug (↑ HbF) –
   (a) Iron (b) Aspirin (c) Hydroxyurea (d) Desferrioxamine
8. Haemophilia A me kaunsa factor deficient?
   (a) VIII (b) XI (c) VII (d) IX
9. Haemophilia inheritance –
   (a) Mitochondrial (b) Autosomal dominant (c) Autosomal recessive (d) X-linked recessive
10. Haemophilia me sabse common bleeding –
    (a) GI bleed (b) Haematuria (c) Epistaxis (d) Haemarthrosis (joints)
11. Sabse common inherited bleeding disorder –
    (a) Haemophilia A (b) ITP (c) Haemophilia B (d) Von Willebrand disease
12. Bachchon me sabse common cancer –
    (a) Retinoblastoma (b) Neuroblastoma (c) ALL (d) Wilms
13. ALL ka confirmatory test –
    (a) Bone marrow aspiration (b) CBC (c) X-ray (d) LFT
14. Leukocoria (white pupillary reflex) –
    (a) Cataract only (b) Glaucoma (c) Conjunctivitis (d) Retinoblastoma
15. Raccoon eyes aur ↑ urinary VMA –
    (a) Leukaemia (b) Wilms (c) Neuroblastoma (d) Lymphoma
16. Sabse common childhood solid tumour –
    (a) Wilms (b) Osteosarcoma (c) Brain tumour (d) Hepatoblastoma
17. Cyclophosphamide ka specific side effect –
    (a) Haemorrhagic cystitis (b) Cardiotoxicity (c) Pulmonary fibrosis (d) Ototoxicity
18. Vincristine ka side effect –
    (a) Hearing loss (b) Haemorrhagic cystitis (c) Red urine (d) Peripheral neuropathy, constipation

Level 2 – Priority & Application

19. Haemophilia bachche ke knee me bleed. Nurse ka first action –
(a) Walk karana (b) Heat lagana (c) RICE + factor replacement jaldi (d) Aspirin

20. Haemophilia bachche me kya avoid karein?
(a) IM injections, aspirin (b) Swimming (c) Factor VIII (d) Paracetamol

21. Sickle cell crisis me sabse important nursing intervention –
(a) Exercise (b) Hydration + analgesia + O_2 (c) Cold compress (d) Fluid restriction

22. Chemotherapy par bachche ka ANC 400/mm^3 aur temperature 38.5 °C. Nurse –
(a) Paracetamol de kar wait (b) Next day check (c) Emergency – cultures, IV antibiotics within 1 hr (d) Discharge

23. Neutropenic bachche ke liye kya NAHI?
(a) Fresh flowers in room (b) Cooked food (c) Reverse isolation (d) Hand hygiene

24. Thalassaemia bachche ko kya avoid karna chahiye?
(a) Vaccines (b) Iron supplements / iron-rich tonic (c) Folic acid (d) Blood transfusion

25. Chemotherapy se mouth ulcers (mucositis) – nurse ki advice –
(a) Alcohol mouthwash (b) Hard toothbrush (c) Spicy food (d) Soft bland diet, saline mouthwash, soft brush

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. ITP ke baare me SAHI NAHI hai –
(a) Viral infection ke baad (b) Mostly self-limiting (c) Platelets low (d) Massive splenomegaly typical

27. Match: P. Thalassaemia Q. Sickle cell R. Haemophilia A S. G6PD — with 1. Factor VIII 2. Fava beans haemolysis 3. HbS 4. ↑ HbF, chelation
(a) P-4, Q-3, R-1, S-2 (b) P-2, Q-3, R-1, S-4 (c) P-4, Q-1, R-3, S-2 (d) P-3, Q-4, R-1, S-2

28. Assertion (A): Thalassaemia major me baar baar transfusion se heart failure ho sakta hai. Reason (R): Iron overload (haemosiderosis) heart me jama hota hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Dono parents thalassaemia carrier hain. Har pregnancy me bachche ke thalassaemia major hone ka chance –
(a) 0 % (b) 25 % (c) 50 % (d) 100 %

30. Bachcha death ko irreversible aur universal kab samajhta hai?
(a) 2 yrs (b) 4 yrs (c) 6 months (d) 9–10 yrs

Answer Key & Explanation

1-c 2–3 months.
2-a ~6 months.
3-c Hair-on-end.
4-d ↑ HbF.
5-b Iron chelation.
6-c Dactylitis.
7-c Hydroxyurea.
8-a Factor VIII.
9-d X-linked recessive.
10-d Haemarthrosis.
11-d vWD.
12-c ALL.
13-a Bone marrow.
14-d Retinoblastoma.
15-c Neuroblastoma.
16-c Brain tumour.
17-a Haemorrhagic cystitis.
18-d Neuropathy, constipation.
19-c RICE + factor.
20-a IM, aspirin avoid.

21-b Hydration first.
22-c Febrile neutropenia – emergency.
23-a Flowers – fungal/ bacterial source.
24-b Iron overload already.
25-d Bland diet, saline rinse.
26-d ITP me spleen palpable nahi typically.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-b AR – 25 %.
30-d 9–10 yrs.

14 Endocrine, Metabolic & Genetic Disorders

Level 1 – Direct

1. Intellectual disability ka sabse common preventable cause –
(a) PKU (b) Down syndrome (c) Congenital hypothyroidism (d) Birth trauma

2. Congenital hypothyroidism ke liye newborn screening –
(a) Urine sugar (b) Bilirubin (c) Hb (d) TSH heel prick 48–72 hrs

3. Congenital hypothyroidism ka earliest sign –
(a) Tachycardia (b) Prolonged jaundice (c) Diarrhoea (d) Tall stature

4. Congenital hypothyroidism me macroglossia aur ___ common hai –
(a) Umbilical hernia (b) Polyuria (c) Weight loss (d) Hyperactivity

5. Levothyroxine dose (newborn) –
(a) 1–2 µg/kg (b) 10–15 µg/kg/day (c) 50 µg/kg (d) 100 µg/kg

6. Type 1 DM me bachche ka common early clue –
(a) Obesity (b) Constipation (c) Secondary enuresis (d) Jaundice

7. DKA me sabse pehle kya dete hain?
(a) Insulin bolus (b) Bicarbonate (c) Potassium (d) IV normal saline

8. Paediatric DKA me death ka common cause –
(a) Hypoglycaemia (b) Cerebral oedema (c) Renal failure (d) Infection

9. Insulin mixing me pehle kaunsa draw karein?
(a) Koi bhi (b) Dono ek saath (c) Clear (regular) (d) Cloudy (NPH)

10. Unconscious hypoglycaemic diabetic child ko –
(a) Insulin (b) Water (c) Oral sugar (d) Glucagon IM / IV dextrose

11. PKU me body odour –
(a) Fishy (b) Maple syrup (c) Musty / mousy (d) Fruity

12. PKU ka screening test –
(a) Schick (b) Guthrie test (c) Mantoux (d) Coombs

13. Galactosaemia me –
(a) Cow milk (b) Breastfeeding contraindicated (c) Normal diet (d) High lactose diet

14. CAH ka sabse common enzyme deficiency –
(a) 21-hydroxylase (b) Aromatase (c) 11-hydroxylase (d) 17-hydroxylase

15. Down syndrome –
(a) Trisomy 13 (b) 45 XO (c) Trisomy 21 (d) Trisomy 18

16. Simian crease aur sandal gap –
(a) Turner (b) Down syndrome (c) Klinefelter (d) Edwards

17. Turner syndrome karyotype –
(a) 46 XY (b) 47 XXX (c) 45 XO (d) 47 XXY

18. Rocker-bottom feet aur clenched fists –
(a) Edwards (trisomy 18) (b) Down (c) Turner (d) Patau

Level 2 – Priority & Application

19. DKA management ke dauran bachche ko headache, bradycardia aur ↓ consciousness. Nurse sochegi –
(a) Cerebral oedema – doctor ko turant, mannitol ready (b) Hypoglycaemia (c) Sleep (d) Normal recovery

20. DKA me insulin start karne ke baad potassium –
(a) Ghatta hai – replacement chahiye (b) Badhta hai (c) Irrelevant (d) Same

21. Diabetic bachcha school me pasina, kaanpna, chakkar – conscious hai. Pehle –
(a) 15 g fast sugar (juice/ glucose), 15 min baad recheck (b) Insulin (c) Exercise (d) Ghar bhejna

22. Insulin injection site ke liye advice –
(a) Only arm (b) Same site daily (c) IM deep (d) Rotate sites

23. Congenital hypothyroid baby ko levothyroxine kaise dein?
(a) Soy formula ke saath (b) Iron syrup ke saath (c) Crush karke thode breast milk/ paani me (d) Weekly

24. Down syndrome newborn ki mother ko pehle kis cheez ke liye prepare karein?
(a) Kuch nahi (b) Adoption (c) Turant surgery (d) Feeding difficulty (hypotonia) aur cardiac evaluation

25. PKU bachche ki diet me kya avoid?
(a) Aspartame aur high protein foods (b) Special formula (c) Vegetables (d) Fruits

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Down syndrome se associated NAHI hai –
(a) Tall stature (b) Leukaemia risk (c) Duodenal atresia (d) Hypotonia

27. Match: P. Turner Q. Klinefelter R. Fragile X S. Cri-du-chat — with 1. Cat-like cry 2. Macroorchidism, large ears 3. Gynaecomastia, tall 4. Webbed neck, coarctation
(a) P-4, Q-2, R-3, S-1 (b) P-1, Q-3, R-2, S-4 (c) P-3, Q-4, R-2, S-1 (d) P-4, Q-3, R-2, S-1

28. Assertion (A): Congenital hypothyroidism me jaldi treatment zaroori hai. Reason (R): Thyroid hormone pehle 2–3 saal me brain development ke liye essential hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Autosomal recessive disorder NAHI hai –
(a) Cystic fibrosis (b) PKU (c) Haemophilia (d) Thalassaemia

30. DKA criteria me SAHI kaunsa?
(a) Only ketonuria (b) Glucose 100, pH 7.4 (c) Glucose > 200, pH < 7.3 / HCO_3 < 15, ketones (d) Glucose < 70

Answer Key & Explanation

1-c Congenital hypothyroidism.
2-d TSH 48–72 hrs.
3-b Prolonged jaundice.
4-a Umbilical hernia.
5-b 10–15 µg/kg/day.
6-c Secondary enuresis.
7-d NS fluids.
8-b Cerebral oedema.
9-c Clear before cloudy.
10-d Glucagon/ IV dextrose.
11-c Musty.
12-b Guthrie.
13-b BF contraindicated.
14-a 21-hydroxylase.
15-c Trisomy 21.
16-b Down.
17-c 45 XO.
18-a Edwards.
19-a Cerebral oedema.
20-a Insulin K^+ ko cells me le jata hai.
21-a Rule of 15.
22-d Rotate – lipohypertrophy.
23-c Small amount milk/ water.
24-d Hypotonia, CHD.

25-a Aspartame = phenylalanine.
26-a Down – short stature.
27-d Sahi.
28-a Dono sahi, R reason hai.
29-c Haemophilia – X-linked.
30-c DKA criteria.

15 Communicable Diseases

Level 1 – Direct

1. Measles ka pathognomonic sign –
   (a) Slapped cheek (b) Forchheimer spots (c) Koplik's spots (d) Strawberry tongue
2. Measles rash pehle kahan dikhta hai?
   (a) Behind ears / hairline (b) Palms (c) Legs (d) Trunk
3. Measles me death ka common cause –
   (a) Diarrhoea only (b) Myocarditis (c) Pneumonia (d) Encephalitis
4. Measles ki late complication (years later) –
   (a) Reye's (b) Orchitis (c) SSPE (d) GBS
5. Chickenpox rash –
   (a) Centrifugal, same stage (b) Only palms (c) Centripetal, pleomorphic, crops (d) Only face
6. Post-auricular lymphadenopathy –
   (a) Rubella (b) Measles (c) Chickenpox (d) Mumps
7. Mumps me parotid swelling ke saath ear lobe –
   (a) Swollen alone (b) Pushed down (c) Normal (d) Pushed up & out
8. Fever utarne ke baad rash –
   (a) Measles (b) Roseola infantum (HHV-6) (c) Scarlet fever (d) Chickenpox
9. Slapped cheek –
   (a) Mumps (b) Parvovirus B19 (c) Measles (d) Rubella
10. Diphtheria ka characteristic –
    (a) Whoop (b) Grey adherent pseudomembrane, bull neck (c) Lockjaw (d) Koplik spots
11. Diphtheria me death ka common cause –
    (a) Renal failure (b) Myocarditis (c) Liver failure (d) Diarrhoea
12. Whooping cough organism –
    (a) Corynebacterium (b) Strep (c) H. influenzae (d) Bordetella pertussis
13. Neonatal tetanus ka first sign –
    (a) Rash (b) Inability to suck (c) Fever (d) Diarrhoea
14. AFP surveillance kis age tak?
    (a) All ages only adults (b) < 18 yrs (c) < 5 yrs (d) < 15 yrs
15. Dengue ka vector –
    (a) Anopheles (b) Culex (c) Sandfly (d) Aedes aegypti
16. Dengue early diagnosis (day 1–5) –
    (a) Blood culture (b) IgG only (c) NS1 antigen (d) Widal
17. India me sabse common malaria –
    (a) P. vivax (b) P. falciparum (c) P. ovale (d) P. malariae
18. Scabies treatment –
    (a) Permethrin 5 % – whole family (b) Antibiotic only (c) Calamine (d) Steroid cream

Level 2 – Priority & Application

19. Chickenpox bachche ko bukhar hai. Mother kaunsi dawa na de?
(a) Antihistamine (b) Aspirin (c) Paracetamol (d) Calamine

20. Measles wale bachche ko kaunsa vitamin dena chahiye?
(a) Vitamin A (b) Vitamin K (c) Vitamin D (d) Vitamin B12

21. Chickenpox bachcha school kab ja sakta hai?
(a) Fever khatam hote hi (b) 2 din baad (c) 1 mahine baad (d) Sabhi lesions crust hone ke baad

22. Dengue me bukhar ke liye –
(a) Paracetamol (b) Aspirin (c) Steroids (d) Ibuprofen

23. Neonatal tetanus baby ki nursing –
(a) Oral feeds forced (b) Bright room, frequent handling (c) Quiet, dark room, minimal handling (d) Bath frequently

24. Dog bite ke baad sabse important first aid –
(a) Wound ko soap & running water se 15 min dhona (b) Stitch turant (c) Tight bandage (d) Chilli/ haldi

25. Pertussis infant me cough ke baad ulti. Nurse –
(a) Only IV lifelong (b) Feeding band (c) Ulti ke baad dobara small feed (d) Lying flat after feed

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Match: P. Measles Q. Rubella R. Mumps S. Scarlet fever — with 1. Sandpaper rash 2. Orchitis 3. Post-auricular nodes 4. Koplik's spots
(a) P-1, Q-3, R-2, S-4 (b) P-4, Q-3, R-2, S-1 (c) P-4, Q-2, R-3, S-1 (d) P-3, Q-4, R-2, S-1

27. Dengue ka warning sign NAHI hai –
(a) Abdominal pain (b) Persistent vomiting (c) Mucosal bleed (d) Mild rash with good intake

28. Assertion (A): HIV-exposed infant ka diagnosis 18 months se pehle antibody test se nahi hota. Reason (R): Mother ki antibodies baby ke blood me 18 months tak reh sakti hain.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Measles infectivity period –
(a) Before prodrome only (b) Only rash period (c) 4 days before to 4 days after rash (d) 2 weeks after rash

30. Vivax malaria radical cure – primaquine kitne din?
(a) 3 days (b) 7 days (c) 14 days (d) 21 days

Answer Key & Explanation

1-c Koplik's spots.
2-a Behind ears.
3-c Pneumonia.
4-c SSPE.
5-c Centripetal, pleomorphic.
6-a Rubella.
7-d Up & out.
8-b Roseola.
9-b Parvovirus B19.
10-b Diphtheria.
11-b Myocarditis.
12-d B. pertussis.
13-b Inability to suck.
14-d < 15 yrs.
15-d Aedes.
16-c NS1.
17-a Vivax.
18-a Permethrin, family.
19-b Aspirin – Reye's.
20-a Vitamin A.
21-d All crusted.
22-a Paracetamol.
23-c Spasms trigger kam.
24-a Washing – most important.

25-c Re-feed.
26-b Sahi.
27-d Mild rash + good intake – warning nahi.
28-a Dono sahi, R reason hai.
29-c 4 days before – 4 after.
30-c 14 days.

16 Musculoskeletal Disorders & Burns

Level 1 – Direct

1. Clubfoot (CTEV) ka gold standard treatment –
   (a) Milwaukee brace (b) Pavlik harness (c) Ponseti method (d) Bryant's traction
2. CTEV me foot kis direction me?
   (a) Up & out (b) Up & in (c) Straight (d) Down & in
3. Ortolani test –
   (a) Spine test (b) Hip dislocate karta hai (c) Knee test (d) Dislocated hip ko relocate karta hai
4. DDH ka treatment (< 6 months) –
   (a) Ponseti cast (b) Bryant's traction (c) Pavlik harness (d) Milwaukee brace
5. DDH kisme zyada common?
   (a) Girls, breech (b) Preterm boys (c) Boys (d) Twins only
6. Bachchon me sabse common fracture –
   (a) Compound (b) Spiral (c) Comminuted (d) Greenstick
7. Supracondylar fracture ki serious complication –
   (a) Volkmann's ischaemic contracture (b) Gangrene foot (c) Non-union (d) Fat embolism
8. 2 saal se kam bachche me femur fracture ke liye traction –
   (a) Russell's (b) Bryant's (gallows) (c) Buck's (d) Halo
9. Acute osteomyelitis ka common organism –
   (a) Strep viridans (b) E. coli (c) Staph aureus (d) TB
10. Scoliosis screening test –
    (a) Ortolani (b) Trendelenburg (c) Adam's forward bend test (d) Thomas test
11. Bachchon ke burns me sabse accurate chart –
    (a) Palm only (b) Lund & Browder (c) Parkland (d) Rule of nines adult
12. Infant me head ka TBSA % (modified rule of nines) –
    (a) 9 % (b) 14 % (c) 18 % (d) 27 %
13. Infant me ek leg ka TBSA % –
    (a) 9 % (b) 14 % (c) 18 % (d) 21 %
14. Bachche ka palm (fingers sahit) –
    (a) 0.1 % (b) 1 % TBSA (c) 5 % (d) 9 %
15. Parkland formula –
    (a) 2 ml × kg (b) 4 ml × kg × % TBSA (c) 20 ml/kg (d) 100 ml/kg
16. Burns resuscitation ka best guide –
    (a) Thirst (b) Skin colour (c) Urine output (d) Temperature
17. Bachchon me sabse common burn –
    (a) Scald (hot liquid) (b) Electrical (c) Radiation (d) Chemical
18. JIA ki important eye complication –
    (a) Glaucoma (b) Cataract only (c) Myopia (d) Uveitis / iridocyclitis

Level 2 – Priority & Application

19. Burn first aid ki sahi advice –
(a) Ice lagana (b) 20 min cool running water (c) Ghee (d) Toothpaste

20. Bryant's traction me buttocks –
(a) Bed se thoda upar (hand-breadth) (b) Lifted 30 cm (c) Bed par flat (d) Pillow par

21. Cast lage bachche me nurse ka sabse important check –
(a) Diet (b) Cast colour (c) Weight (d) Neurovascular status – colour, warmth, CRT, movement of toes

22. Traction ke weights –
(a) Raat ko hata do (b) Floor par rakh do (c) Bed par (d) Freely hang karein, kabhi hata na

23. Supracondylar fracture bachche ko passive finger extension par severe pain. Suspect –
(a) Compartment syndrome (b) Nerve recovery (c) Normal (d) Infection

24. Burns child ko IV fluids kab chahiye?
(a) Kabhi nahi (b) Kisi bhi burn me (c) > 10 % TBSA (d) > 50 % only

25. Hip spica cast bachche ki care –
(a) Cast geela (b) Turning nahi (c) Abduction bar se uthana (d) 2-hrly turning, perineum ko plastic se protect

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. 10 kg bachcha, 20 % burns. Parkland – pehle 8 ghante me fluid –
(a) 200 ml (b) 400 ml (c) 800 ml (d) 1600 ml

27. Match: P. CTEV Q. DDH R. Scoliosis S. Femur fracture < 2 yrs — with 1. Bryant's traction 2. Milwaukee brace 3. Pavlik harness 4. Ponseti
(a) P-4, Q-3, R-2, S-1 (b) P-4, Q-2, R-3, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-3, Q-4, R-2, S-1

28. Assertion (A): Bachchon me fractures jaldi heal hote hain. Reason (R): Bachchon me periosteum mota aur blood supply achhi hoti hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. Child abuse ka fracture pattern NAHI hai –
(a) Spiral fracture in non-ambulant infant (b) Single distal radius buckle fracture after fall while running (c) Posterior rib fracture (d) Metaphyseal corner fracture

30. TBSA calculation me kaunsa burn include NAHI karte?
(a) Full thickness (b) Partial thickness (c) Superficial (1st degree) (d) Deep dermal

Answer Key & Explanation

1-c Ponseti.
2-d Down & in.
3-d Relocates.
4-c Pavlik.
5-a Girls, breech.
6-d Greenstick.
7-a Volkmann's.
8-b Bryant's.
9-c Staph aureus.
10-c Adam's test.
11-b Lund & Browder.
12-c 18 %.
13-b 14 %.
14-b 1 %.
15-b 4 ml × kg × %.
16-c Urine output.
17-a Scald.
18-d Uveitis.
19-b Cool running water 20 min.
20-a Just off bed – counter-traction.
21-d Neurovascular check.
22-d Hang freely.
23-a Compartment syndrome.
24-c > 10 % children.

25-d Turning, perineal protection.
26-b 4 × 10 × 20 = 800; half = 400 ml.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-b Common accidental injury.
30-c Superficial excluded.

17 Behavioural Disorders, Child Abuse & Poisoning

Level 1 – Direct

1. Temper tantrums kis age me sabse common?
   (a) Adolescent (b) Toddler (c) School age (d) Infant
2. Thumb sucking normal kis age tak?
   (a) 1 yr (b) 4 yrs (c) 8 yrs (d) 12 yrs
3. Pica kis se associated hai?
   (a) Diabetes (b) Vitamin A (c) Obesity (d) Iron deficiency / lead poisoning
4. ADHD ka drug of choice –
   (a) Diazepam (b) Phenytoin (c) Haloperidol (d) Methylphenidate
5. Autism ki pehchaan –
   (a) Hyperactivity only (b) Rash (c) Poor eye contact, repetitive behaviour (d) High fever
6. Mild intellectual disability IQ –
   (a) 50–69 (b) < 20 (c) 35–49 (d) 20–34
7. IQ formula –
   (a) MA/CA × 100 (b) MA × CA (c) CA/MA × 100 (d) MA + CA
8. Child abuse ka sabse common type –
   (a) Sexual (b) Physical (c) Neglect (d) Emotional
9. Shaken baby syndrome me –
   (a) Burns (b) Fever (c) Fracture femur only (d) Subdural haematoma + retinal haemorrhage
10. Most accident-prone age group –
    (a) Infant (b) Adolescent (c) School age (d) Toddler
11. India me bachchon me sabse common poisoning –
    (a) Kerosene (b) Lead (c) Mushroom (d) Paracetamol
12. Paracetamol poisoning ka antidote –
    (a) Naloxone (b) Atropine (c) N-acetylcysteine (d) Desferrioxamine
13. Iron poisoning ka antidote –
    (a) NAC (b) Vitamin K (c) Desferrioxamine (d) Flumazenil
14. Organophosphate poisoning –
    (a) Atropine + pralidoxime (b) Calcium (c) NAC (d) Naloxone
15. Opioid poisoning antidote –
    (a) Glucagon (b) Flumazenil (c) Naloxone (d) Atropine
16. SIDS ka peak age –
    (a) 2 years (b) Newborn day 1 (c) 2–4 months (d) 1 year
17. Girls me puberty ka first sign –
    (a) Height spurt end (b) Thelarche (breast budding) (c) Pubic hair (d) Menarche

18. Activated charcoal ka dose –
(a) 0.1 g/kg (b) 1 g/kg (c) 10 g/kg (d) 50 g fixed for all

? Level 2 – Priority & Application

19. 2 saal ka bachcha supermarket me tantrum karta hai. Advice –
(a) Turant chocolate de do (b) Daantna loudly (c) Maarna (d) Safe ho to ignore, shaant raho, consistent raho

20. Bachche ne kerosene pi liya. Mother kya NA kare?
(a) Hospital le jaye (b) Ulti karwaye (c) Container saath le jaye (d) Shaant rakhe

21. Nurse ko bachche ke body par alag-alag rang ke bruises aur cigarette burns dikhte hain. Pehla step –
(a) Ignore (b) Document karke authorities ko report, child safety (c) Discharge (d) Parents par chillana

22. SIDS prevention ki advice –
(a) Pet ke bal sulana (b) Bed sharing (c) Peeth ke bal (supine), firm mattress, no soft toys (d) Pillow aur blanket zyada

23. Corrosive acid peene wale bachche me –
(a) Ulti karwana (b) Na ulti, na lavage; airway & referral (c) Neutralise with baking soda (d) Gastric lavage

24. ADHD bachche ke classroom ke liye –
(a) Free roam (b) Long lectures (c) Front seat, short structured tasks (d) Peeche ki seat

25. Adolescent ka interview (HEEADSSS) –
(a) Parents ke saamne hi (b) Confidential, akele, non-judgmental (c) Phone par (d) Group me

? Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. Activated charcoal kis poisoning me useful NAHI?
(a) Iron (b) Phenobarbitone (c) Paracetamol (d) Carbamazepine

27. Match: P. Paracetamol Q. Iron R. Organophosphate S. Benzodiazepine — with 1. Flumazenil 2. Atropine 3. Desferrioxamine 4. NAC
(a) P-4, Q-3, R-2, S-1 (b) P-4, Q-2, R-3, S-1 (c) P-1, Q-3, R-2, S-4 (d) P-3, Q-4, R-2, S-1

28. Assertion (A): Kerosene poisoning me ulti nahi karwate. Reason (R): Ulti se aspiration pneumonitis ho sakta hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. 4 saal ke bachche ke liye time-out (1 min per year) –
(a) 1 min (b) 4 min (c) 10 min (d) 30 min

30. Mental age 6 saal, chronological age 10 saal. IQ –
(a) 40 (b) 60 (c) 80 (d) 166

✓ Answer Key & Explanation

1-b Toddler.
2-b 4 yrs.
3-d Iron deficiency/ lead.
4-d Methylphenidate.
5-c Social deficit + repetitive.
6-a 50–69.
7-a MA/CA × 100.
8-c Neglect.
9-d SDH + retinal haemorrhage.
10-d Toddler.
11-a Kerosene.
12-c NAC.

13-c Desferrioxamine.
14-a Atropine + PAM.
15-c Naloxone.
16-c 2–4 months.
17-b Thelarche.
18-b 1 g/kg.
19-d Ignore, consistent.
20-b No induced vomiting.
21-b Document, report, protect.
22-c Back to sleep.
23-b No emesis/ lavage.
24-c Front seat, structure.
25-b Confidential.
26-a Iron charcoal se bind nahi hota.
27-a Sahi.
28-a Dono sahi, R reason hai.
29-b 4 min.
30-b 6/10 × 100 = 60.

18 Paediatric Emergencies & Life Support

Level 1 – Direct

1. Paediatric CPR ka sequence (AHA) –
   (a) C-A-B (b) A-C-B (c) B-A-C (d) A-B-C
2. Infant me pulse check –
   (a) Carotid (b) Femoral only (c) Brachial (d) Radial
3. Infant me compression depth –
   (a) 2 cm (b) 4 cm (c) 6 cm (d) 8 cm
4. Child me compression depth –
   (a) 3 cm (b) 5 cm (c) 7 cm (d) 9 cm
5. Two rescuer child CPR ratio –
   (a) 3 : 1 (b) 5 : 1 (c) 15 : 2 (d) 30 : 2
6. Compression rate –
   (a) 60–80/min (b) 80–100/min (c) 100–120/min (d) 140–160/min
7. Bachchon me cardiac arrest ka sabse common cause –
   (a) Hypoxia / respiratory failure (b) Electrolyte (c) Trauma always (d) Primary heart disease
8. Child me shock ka earliest sign –
   (a) Tachycardia (b) Hypotension (c) Coma (d) Bradycardia
9. Shock me fluid bolus –
   (a) 5 ml/kg (b) 20 ml/kg (c) 50 ml/kg (d) 100 ml/kg
10. Cardiac arrest me adrenaline dose –
    (a) 0.01 mg/kg IV (0.1 ml/kg of 1:10,000) (b) 0.1 mg/kg of 1:1000 IV (c) 1 mg/kg (d) 10 mg fixed
11. Defibrillation first shock –
    (a) 0.5 J/kg (b) 2 J/kg (c) 10 J/kg (d) 360 J
12. Uncuffed ET tube size formula –
    (a) (Age/2) + 2 (b) Age + 4 (c) Age × 2 (d) (Age/4) + 4
13. IV fail hone par emergency access –
    (a) Scalp vein (b) Subclavian always (c) Umbilical in 5-yr child (d) Intraosseous – proximal tibia
14. Anaphylaxis me first drug –
    (a) IM adrenaline (b) Antihistamine (c) Hydrocortisone (d) Salbutamol
15. AVPU me "P" ka matlab –
    (a) Pupils (b) Pallor (c) Pulse (d) Responds to pain
16. Head box se oxygen flow –
    (a) 0.5 L/min (b) 1 L/min (c) 5–7 L/min (d) 15 L/min
17. Raised ICP me head position –
    (a) Turned to side, flexed (b) 15–30° elevated, midline (c) Flat (d) Trendelenburg
18. Infant choking (conscious, can't cough) –
    (a) Blind finger sweep (b) Abdominal thrusts (c) Water (d) 5 back blows + 5 chest thrusts

Level 2 – Priority & Application

19. Emergency department me pehle kis bachche ko dekhenge?
(a) 2 saal ka convulsing now (b) 3 saal ka cough-cold (c) 5 saal ka rash (d) 6 saal ka ear pain

20. Unresponsive infant – breathing nahi, pulse nahi. Akele nurse kya kare?
(a) 2 min CPR phir help call (b) Wait (c) Only breaths 10 min (d) Pehle phone

21. Infant ka airway kholte waqt –
(a) Turn head fully (b) Hyperextend (c) Neutral / sniffing position (d) Flex chin to chest

22. 1 saal se bade conscious choking child ke liye –
(a) Abdominal thrusts (Heimlich) (b) Finger sweep (c) Back blows only (d) Chest thrusts only

23. Child me hypotension kis stage ka sign hai?
(a) Normal (b) Recovery (c) Early compensated (d) Late decompensated

24. Vaccine ke baad anaphylaxis – adrenaline kahan aur kaise?
(a) IV bolus 1:1000 (b) Oral (c) IM anterolateral thigh 0.01 mg/kg of 1:1000 (d) SC deltoid 1 ml

25. Unconscious child ki eye care –
(a) Eyes open rakho (b) Kuch nahi (c) Hot compress (d) Saline / lubricant, eyelids close tape

Level 3 – Advanced (NOT / Match / Assertion-Reason / Calculation)

26. 20 kg bachche ko shock me fluid bolus (20 ml/kg) –
(a) 200 ml (b) 400 ml (c) 600 ml (d) 800 ml

27. 4 saal ke bachche ka uncuffed ET tube size –
(a) 4 mm (b) 5 mm (c) 6 mm (d) 7 mm

28. Assertion (A): Infant CPR me abdominal thrusts nahi dete. Reason (R): Infant ka liver bada aur protected nahi hota, injury ka risk hai.
(a) Both A and R true, R explains A (b) Both A and R true, R does not explain A (c) A true, R false (d) A false, R true

29. ETAT emergency sign NAHI hai –
(a) Convulsing now (b) Shock (c) Mild fever with cough (d) Obstructed breathing

30. Match: P. Nasal prongs Q. Head box R. Non-rebreathing mask S. CPAP — with 1. 10–15 L/min 2. 5–7 L/min 3. Neonatal RDS 4. 0.5–2 L/min
(a) P-4, Q-1, R-2, S-3 (b) P-2, Q-4, R-1, S-3 (c) P-4, Q-2, R-1, S-3 (d) P-3, Q-2, R-1, S-4

Answer Key & Explanation

1-a C-A-B.
2-c Brachial.
3-b 4 cm.
4-b 5 cm.
5-c 15 : 2.
6-c 100–120.
7-a Hypoxia.
8-a Tachycardia.
9-b 20 ml/kg.
10-a 0.01 mg/kg.
11-b 2 J/kg.
12-d (Age/4) + 4.
13-d IO – proximal tibia.
14-a IM adrenaline.
15-d Pain.
16-c 5–7 L/min.
17-b 15–30°, midline.
18-d Back blows + chest thrusts.
19-a Convulsing now – emergency.
20-a Unwitnessed – 2 min CPR first.
21-c Neutral – large occiput.
22-a Heimlich.
23-d Late sign.
24-c IM thigh.
25-d Corneal protection.
26-b 20 × 20 = 400 ml.

27-b 4/4 + 4 = 5 mm.
28-a Dono sahi, R reason hai.
29-c Priority/ queue.
30-c Sahi.

19 GSSSB Pattern Mock Test – 100 MCQs (Full Child Health Syllabus)



Mock test ke rules

100 questions · 100 marks · 75 minutes (Child Health section ka practice). Galat answer par **−0.25**. Timer lagao, alag kagaz par answer likho, phir answer key se milao.

Score guide: 80+ Excellent · 65–79 Good · 50–64 Weak topics revise karo · < 50 Notes dobara padho.

? Mock Test – Part A (1–50)

1. Infancy ki age –
(a) 1–3 years (b) Birth – 28 days (c) 3–6 years (d) Birth – 1 year

2. IMR kis per express hota hai?
(a) 100 births (b) 1000 population (c) 1000 live births (d) 1,00,000 live births

3. Child helpline –
(a) 102 (b) 108 (c) 1091 (d) 1098

4. RBSK ka focus –
(a) 4 D's screening (b) Family planning (c) Elderly care (d) Adolescent nutrition

5. ICDS ki worker –
(a) ANM (b) Anganwadi worker (c) MPW male (d) Staff nurse

6. Weight triples at –
(a) 1 year (b) 2 years (c) 3 years (d) 6 months

7. Anterior fontanelle closes –
(a) 3 months (b) 12–18 months (c) 6–8 weeks (d) 3 years

8. First milk tooth –
(a) Upper canine (b) First molar (c) Lower central incisor (d) Second molar

9. Sits without support –
(a) 4 months (b) 8 months (c) 12 months (d) 15 months

10. Pincer grasp –
(a) 4 months (b) 6 months (c) 9–10 months (d) 18 months

11. Copies a triangle –
(a) 2 years (b) 3 years (c) 4 years (d) 5 years

12. Erikson – school age –
(a) Identity vs role confusion (b) Initiative vs guilt (c) Trust vs mistrust (d) Industry vs inferiority

13. Toddler play –
(a) Parallel (b) Cooperative (c) Solitary only (d) Competitive

14. Stunting indicates –
(a) Chronic malnutrition (b) Acute malnutrition (c) Dehydration (d) Obesity

15. Exclusive breastfeeding –
(a) 1 year (b) 3 months (c) 4 months (d) 6 months

16. SAM MUAC –
(a) < 11.5 cm (b) < 12.5 cm (c) < 13.5 cm (d) < 14 cm

17. Kwashiorkor hallmark –
(a) Oedema (b) Severe wasting (c) Old man face (d) Good appetite

18. F-75 is used in –
(a) Obesity (b) Rehabilitation (c) Follow-up (d) Stabilisation phase

19. Night blindness – vitamin –
(a) C (b) D (c) A (d) K

20. Earliest sign of rickets –
(a) Pigeon chest (b) Craniotabes (c) Bow legs (d) Harrison's sulcus

21. BCG route –
(a) Intramuscular (b) Subcutaneous (c) Intradermal (d) Oral

22. MR vaccine route –
(a) Oral (b) Intravenous (c) Subcutaneous (d) Intradermal

23. Pentavalent contains –
(a) BCG + OPV (b) DPT + IPV + MR (c) Td + Hep A (d) DPT + Hep B + Hib

24. Most heat-sensitive vaccine –
(a) DPT (b) OPV (c) Hep B (d) Td

25. Test for freezing –
(a) Schick (b) Mantoux (c) VVM (d) Shake test

26. ILR temperature –
(a) 0 °C (b) +15 °C (c) +2 to +8 °C (d) −15 to −25 °C

27. Separation anxiety first stage –
(a) Protest (b) Denial (c) Despair (d) Detachment

28. FLACC scale age –
(a) Adolescents (b) 2 months – 7 yrs (c) Preterm only (d) Adults

29. Preferred IM site in infant –
(a) Dorsogluteal (b) Deltoid (c) Forearm (d) Vastus lateralis

30. Ear drops in child < 3 yrs – pinna –
(a) Up & forward (b) Straight (c) Down & back (d) Up & back

31. Maintenance fluid first 10 kg –
(a) 20 ml/kg/day (b) 50 ml/kg/day (c) 100 ml/kg/day (d) 150 ml/kg/day

32. Preterm –
(a) < 28 weeks (b) < 34 weeks (c) < 37 weeks (d) < 40 weeks

33. Main heat loss at birth –
(a) Conduction (b) Radiation (c) Convection (d) Evaporation

34. Pathological jaundice –
(a) Peak day 4 (b) After 24 hrs (c) Within 24 hrs (d) Gone by day 10

35. Phototherapy – eyes –
(a) Covered (b) Taped open (c) Not relevant (d) Open

36. RDS cause –
(a) Meconium (b) Heart defect (c) Infection (d) Surfactant deficiency

37. NEC sign –
(a) Double bubble (b) Steeple (c) Thumb (d) Pneumatosis intestinalis

38. IMNCI pink –
(a) OPD treatment (b) Urgent referral (c) Home care (d) Follow-up only

39. Fast breathing 2–12 months –
(a) ≥ 30/min (b) ≥ 40/min (c) ≥ 50/min (d) ≥ 60/min

40. Diarrhoea –
(a) 5 formed stools (b) ≥ 3 loose stools in 24 hrs (c) Any stool (d) 1 loose stool

41. Plan B ORS –
(a) 20 ml/kg bolus (b) 50 ml/kg in 1 hr (c) 75 ml/kg in 4 hrs (d) 100 ml/kg IV

42. Low-osmolarity ORS –
(a) 200 mOsm/L (b) 245 mOsm/L (c) 311 mOsm/L (d) 350 mOsm/L

43. Zinc > 6 months –
(a) 5 mg × 30 days (b) 10 mg × 7 days (c) 20 mg × 14 days (d) 40 mg × 3 days

44. Croup cause –
(a) Hib (b) Pneumococcus (c) Parainfluenza (d) RSV

45. Epiglottitis – nursing –
(a) Throat swab immediately (b) Tongue depressor (c) Don't examine throat (d) Lay supine

46. Bronchiolitis cause –
(a) Staph (b) Hib (c) TB (d) RSV

47. Foreign body commonly in –
(a) Left main bronchus (b) Oesophagus only (c) Right main bronchus (d) Larynx always

48. Cystic fibrosis test –
(a) Sweat chloride (b) ABG (c) Spirometry (d) Mantoux

49. Most common CHD –
(a) ASD (b) VSD (c) TOF (d) PDA

50. Boot-shaped heart –
(a) PDA (b) TAPVC (c) TOF (d) TGA

? Mock Test – Part B (51–100)

51. Machinery murmur –
(a) Coarctation (b) VSD (c) PDA (d) ASD

52. Tet spell position –
(a) Supine (b) Knee-chest (c) Prone (d) Trendelenburg

53. Withhold digoxin in infant if apical pulse –
(a) < 60/min (b) < 90–110/min (c) < 140/min (d) < 160/min

54. Rheumatic fever follows –
(a) Viral fever (b) Strep throat (c) Staph skin (d) UTI

55. RF secondary prophylaxis –
(a) Aspirin daily lifelong (b) Amoxicillin weekly (c) Benzathine penicillin every 21 days (d) Steroids monthly

56. Kawasaki complication –
(a) Renal failure (b) Stroke (c) Hepatitis (d) Coronary aneurysm

57. Cleft lip repair age –
(a) 2 years (b) 5 years (c) ~3 months (d) Birth

58. Cleft lip post-op position –
(a) Sitting 90° (b) Trendelenburg (c) Prone (d) Supine / side

59. TEF 3 C's –
(a) Cry, colic, constipation (b) Coughing, choking, cyanosis (c) Convulsion, coma, cyanosis (d) Cold, cough, cramps

60. Pyloric stenosis vomiting –
(a) Faecal (b) Projectile non-bilious (c) Bilious (d) Bloody

61. Red currant jelly stool –
(a) Hirschsprung (b) Intussusception (c) Pyloric stenosis (d) Coeliac

62. Hirschsprung diagnosis –
(a) USG (b) Rectal biopsy (c) Barium meal (d) Stool culture

63. Omphalocele –
(a) No sac (b) Covered by sac (c) Diaphragmatic (d) Inguinal

64. Coeliac disease – avoid –
(a) Fruits (b) Maize (c) Wheat, barley, rye (d) Rice

65. Nephrotic syndrome commonest type –
(a) PSGN (b) Membranous (c) Minimal change (d) FSGS

66. Nephrotic best fluid indicator –
(a) Thirst (b) BP (c) Urine colour (d) Daily weight

67. AGN urine –
(a) Cola coloured (b) Clear (c) Milky (d) Frothy

68. UTI organism –
(a) Staph (b) Pseudomonas (c) E. coli (d) Candida

69. Wilms tumour –
(a) Massage (b) Heat pack (c) Palpate daily (d) Don't palpate abdomen

70. Hypospadias –
(a) Circumcise early (b) Avoid circumcision (c) Orchidopexy (d) No treatment

71. Enuresis diagnosed after –
(a) 2 years (b) 3 years (c) 5 years (d) 10 years

72. Hydrocephalus earliest sign (infant) –
(a) Papilloedema (b) Headache (c) Rapid increase in OFC (d) Diplopia

73. Meningomyelocele position –
(a) Sitting (b) Side on sac (c) Prone (d) Supine

74. Infant meningitis sign –
(a) Jaundice (b) Sunken fontanelle (c) Bulging fontanelle (d) Small head

75. Simple febrile seizure –
(a) < 15 min, generalised (b) > 30 min (c) Focal (d) Recurrent in a day

76. Status epilepticus first drug –
(a) Phenytoin (b) Benzodiazepine (c) Valproate oral (d) Carbamazepine

77. Commonest CP –
(a) Spastic (b) Athetoid (c) Mixed (d) Ataxic

78. Reye's syndrome –
(a) Aspirin + viral illness (b) Insulin (c) Paracetamol overdose (d) Iron

79. Thalassaemia X-ray –
(a) Snowstorm (b) Hair-on-end (c) Boot shape (d) Pepper-pot

80. Sickle crisis – priority –
(a) Cold compress (b) Exercise (c) Fluid restriction (d) Hydration

81. Haemophilia A –
(a) Factor VIII (b) Platelets (c) Factor VII (d) Factor IX

82. Commonest childhood cancer –
(a) Retinoblastoma (b) ALL (c) Osteosarcoma (d) Wilms

83. Leukocoria –
(a) Retinoblastoma (b) Glaucoma (c) Wilms (d) Neuroblastoma

84. Congenital hypothyroidism screening –
(a) Urine iodine (b) T3 at birth (c) TSH at 48–72 hrs (d) Bone age

85. DKA – first step –
(a) Insulin bolus (b) IV normal saline (c) Bicarbonate (d) Potassium

86. PKU odour –
(a) Maple syrup (b) Fishy (c) Musty (d) Fruity

87. Down syndrome –
(a) Trisomy 18 (b) 45 XO (c) Trisomy 21 (d) 47 XXY

88. Koplik's spots –
(a) Rubella (b) Measles (c) Chickenpox (d) Mumps

89. Chickenpox – avoid –
(a) Paracetamol (b) Aspirin (c) Calamine (d) Antihistamine

90. Diphtheria death due to –
(a) Renal failure (b) Myocarditis (c) Hepatitis (d) Diarrhoea

91. Neonatal tetanus first sign –
(a) Inability to suck (b) Rash (c) Vomiting (d) Fever

92. Dengue vector –
(a) Anopheles (b) Aedes (c) Sandfly (d) Culex

93. Scabies treatment –
(a) Antibiotic (b) Permethrin 5 % – whole family (c) Antifungal (d) Steroid

94. Clubfoot treatment –
(a) Milwaukee brace (b) Pavlik harness (c) Ponseti (d) Bryant's traction

95. DDH test (relocates) –
(a) Ortolani (b) Thomas (c) Barlow (d) Trendelenburg

96. Infant head TBSA –
(a) 9 % (b) 14 % (c) 18 % (d) 27 %

97. Kerosene poisoning –
(a) Ipecac (b) Activated charcoal always (c) Gastric lavage (d) Don't induce vomiting

98. Paracetamol antidote –

(a) Naloxone (b) Desferrioxamine (c) Atropine (d) N-acetylcysteine

99. Infant CPR – pulse check –

(a) Radial (b) Carotid (c) Temporal (d) Brachial

100. Earliest sign of shock in child –

(a) Bradycardia (b) Coma (c) Hypotension (d) Tachycardia

Answer Key – Mock Test (with explanation)

1-d Birth – 1 yr.
2-c Per 1000 live births.
3-d 1098.
4-a 4 D's.
5-b AWW.
6-a 1 yr.
7-b 12–18 months.
8-c Lower central incisor.
9-b 8 months.
10-c 9–10 months.
11-d 5 yrs.
12-d Industry vs inferiority.
13-a Parallel.
14-a Chronic.
15-d 6 months.
16-a < 11.5 cm.
17-a Oedema.
18-d Stabilisation.
19-c Vitamin A.
20-b Craniotabes.
21-c Intradermal.
22-c Subcutaneous.
23-d DPT + Hep B + Hib.
24-b OPV.
25-d Shake test.
26-c +2 to +8 °C.
27-a Protest.
28-b 2 m–7 y.
29-d Vastus lateralis.
30-c Down & back.
31-c 100 ml/kg.
32-c < 37 wks.
33-d Evaporation.
34-c < 24 hrs.
35-a Covered.
36-d Surfactant.
37-d Pneumatosis.
38-b Referral.
39-c ≥ 50.
40-b ≥ 3/24 hrs.
41-c 75 ml/kg in 4 hrs.
42-b 245.
43-c 20 mg × 14 d.
44-c Parainfluenza.
45-c Don't examine throat.
46-d RSV.
47-c Right bronchus.
48-a Sweat chloride.
49-b VSD.
50-c TOF.
51-c PDA.
52-b Knee-chest.
53-b < 90–110.
54-b Strep throat.
55-c Every 21 days.
56-d Coronary aneurysm.
57-c ~3 months.
58-d Supine/ side.
59-b Coughing, choking, cyanosis.
60-b Projectile non-bilious.
61-b Intussusception.
62-b Rectal biopsy.
63-b Sac.
64-c Gluten grains.
65-c Minimal change.
66-d Daily weight.
67-a Cola.
68-c E. coli.
69-d Don't palpate.
70-b Avoid circumcision.
71-c 5 yrs.
72-c ↑ OFC.
73-c Prone.
74-c Bulging fontanelle.
75-a < 15 min generalised.
76-b Benzodiazepine.
77-a Spastic.
78-a Aspirin + virus.
79-b Hair-on-end.
80-d Hydration.
81-a Factor VIII.
82-b ALL.
83-a Retinoblastoma.
84-c TSH 48–72 hrs.
85-b NS fluids.
86-c Musty.
87-c Trisomy 21.
88-b Measles.

89-b Aspirin.
90-b Myocarditis.
91-a Inability to suck.
92-b Aedes.
93-b Permethrin.
94-c Ponseti.
95-a Ortolani.
96-c 18 %.
97-d No emesis.
98-d NAC.
99-d Brachial.
100-d Tachycardia.

— Child Health Nursing · GSSSB Staff Nurse Preparation · Padhte raho, revise karte raho! —